

The Health Fix

An AuDHD Woman's Guide to
Managing Your Body and Mind

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For every woman who has put her own health
at the bottom of the list —
your body is not an afterthought.
It is the system you live in,
and it deserves the same care you give everything else.

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Foreword: Your Body Is Part of the System

IF YOU HAVE AuDHD, YOU PROBABLY HAVE A COMPLICATED RELATIONSHIP WITH YOUR BODY. MAYBE YOU FORGET TO EAT UNTIL YOU'RE SHAKING. MAYBE YOU CAN'T SLEEP BECAUSE YOUR BRAIN WON'T STOP. MAYBE YOU PUT OFF THE DENTIST FOR FOUR YEARS. MAYBE YOU HAVE TEN OVERDUE PRESCRIPTIONS AND A PILE OF UNOPENED MAIL FROM THE SURGERY.

This is not laziness. This is what happens when the executive function required to manage a human body — remembering appointments, refilling prescriptions, noticing symptoms, prioritising your own wellbeing — is the exact function your brain struggles with most.

And yet, your body is the system you live in. When it breaks down, everything else breaks down. Burnout, overwhelm, sensory overload — they all have physical consequences that compound the mental ones. The AuDHD woman who ignores her health is not just putting off a check-up; she is pouring fuel on a fire that is already burning.

This book is about managing your health as an AuDHD woman — not with guilt, not with another impossible routine, but with systems designed for your actual brain.

Sleep, food, movement, hormones, medication, medical appointments: each gets a chapter, and each gets a neurodivergent-friendly approach.

Your body is not the enemy. It is not a distraction from your real life. It is your real life's infrastructure — and it's time to give it the same care you give everything else.

Toolkit Summary

- Health management is an executive function task, which makes it doubly hard for AuDHD women.
- Neglecting your body compounds burnout and overwhelm — they're connected.
- The solution is systems, not guilt or impossible routines.
- Your body is the infrastructure of your life. It deserves maintenance.

Chapter 1: Why AuDHD Women Neglect Their Health

THERE'S A TOOTH THAT'S BEEN HURTING FOR SIX MONTHS. NOT CONSTANTLY — JUST SOMETIMES, WHEN YOU EAT SOMETHING COLD, A SHARP REMINDER THAT YOU SHOULD PROBABLY DO SOMETHING ABOUT IT. YOU'VE THOUGHT ABOUT BOOKING AN APPOINTMENT MAYBE FORTY TIMES. YOU'VE OPENED THE DENTAL PRACTICE'S WEBSITE TWICE. ONCE YOU EVEN GOT AS FAR AS THE BOOKING PAGE BEFORE A NOTIFICATION PULLED YOU AWAY, AND THEN THE MOMENT WAS GONE, AND THE TOOTH WENT BACK TO BEING A BACKGROUND PROBLEM YOU CARRY AROUND LIKE A PEBBLE IN YOUR SHOE. YOU WILL DEAL WITH IT LATER. LATER NEVER COMES, BUT THE TOOTH IS STILL THERE, AND SO IS THE QUIET SHAME ABOUT THE TOOTH — AND UNDERNEATH IT, THE WEARY KNOWLEDGE THAT THIS IS NOT THE ONLY THING YOU'RE NEGLECTING. THERE'S THE SMEAR TEST POSTPONED FOR TWO YEARS, THE PRESCRIPTION NOT REFILLED IN THREE WEEKS, THE LUMP YOU NOTICED AND DECIDED NOT TO THINK ABOUT, THE MIGRAINE THAT'S BEEN GETTING WORSE AND THAT YOU'VE STARTED TO ACCEPT AS SIMPLY HOW LIFE IS NOW.

If that paragraph made you flinch, you're in the right place. This chapter is about the pattern underneath it — why AuDHD women neglect their health in ways that are completely predictable and completely unfair, and why

the standard explanations ("you're lazy", "you don't care about yourself") have cost you years of needless shame. The truth is stranger than the shame suggests. You don't neglect your health because you don't value it. You neglect it because your brain is structurally bad at the exact set of tasks that keeping a human body alive requires. None of this is your fault. All of it is fixable — but only once you stop trying to fix it with guilt and start fixing it with systems. That's what this book is for. Let's look at the pattern first, because it has a shape.

The Shape of the Neglect

Let's name the patterns, because naming them removes some of their power. The first is forgetting to eat — not occasionally, but as a way of life. You get up, you have coffee, you start something, and at four in the afternoon you realise you've eaten one slice of toast and a packet of crisps since yesterday. The hunger signals arrived at some point, but they were background noise, and your brain filed them under "deal with later" — and later never came, because by the time you noticed, the hunger had passed into a hollow, shaky state that doesn't feel like hunger at all. It feels like irritability, like being unable to make decisions, like crying at an advert for dog food. And then you eat, and the fog lifts, and you wonder how you could have forgotten something as basic as food — again.

The second pattern is postponing appointments. Not because you're afraid of the dentist or the doctor — or not only because of that. Because booking the appointment requires a phone call, and a phone call requires finding the number, and finding the number requires opening the browser, and opening the browser means encountering seventeen other tabs, and somewhere between the phone call and the browser the whole task collapses under its own weight. So the appointment doesn't get made, and the reminder letter goes into the pile, and the pile becomes a monument to everything you haven't done.

The third pattern is ignoring symptoms. You notice something — a pain, a lump, a change, a bleed that isn't normal — and your first instinct is not to investigate but to wait. Not because you're in denial, exactly. Because symptoms are ambiguous, and ambiguity is a trap. If you go to the doctor and it's nothing, you've spent energy, money and a morning in a fluorescent waiting room for nothing. If you go and it's something, you have to deal with it. Both options feel worse than the current option, which is to file the symptom under "probably fine" and carry on. So you carry on, and the symptom gets louder, and eventually it stops being a question you can ignore and becomes a crisis you have to handle — at which point it's bigger, scarier and more expensive than it ever needed to be.

And the fourth pattern is the phrase that rules all the others: "I'll deal with it later." Later is where health goes to die. It's a promise that dissolves the moment you leave

the room — a deferral, and for an AuDHD brain a deferral is usually permanent, because "later" has no anchor. There's no moment when it arrives. Just the horizon, always receding, always one more day away.

Why Your Brain Does This

Now the important part: none of this is because you don't care. Let's walk through the actual mechanisms, because understanding them is the difference between fighting yourself for the rest of your life and finally building something that works.

Executive dysfunction. Your brain's executive function — the management team that plans, prioritises, initiates and follows through — is chronically understaffed. And here's the crux of this book: every single health task is an executive function task. Booking an appointment requires planning, initiating and follow-through. Refilling a prescription requires noticing you're running out (prospective memory), remembering to do it (working memory), and actually doing it (initiation). Even "eat lunch" requires noticing hunger, deciding what to eat, preparing food and sitting down — four executive steps, any one of which can break the chain. When someone says "just make the appointment", they're describing one step; for you it's a chain of fifteen, and chains break at the weakest link. That's why "just do it" advice never worked: it assumes the chain exists.

Time blindness. You don't experience time the way other people do. "Later today", "next week" and "six months ago" all live in the same foggy, undifferentiated now. This is why appointments surprise you, why you run out of medication, why you don't notice that you haven't been to the dentist in four years — because four years doesn't feel like four years. It feels like "a while ago, I should probably go sometime." Time blindness doesn't just make you late. It makes consequences invisible. And invisible consequences are extremely easy to ignore — which is why the toothache never quite reaches the top of the list.

The sensory minefield. Here's the part that people who don't share your neurotype never understand: for an AuDHD woman, especially one with an autistic sensory system, a medical appointment is not a neutral transaction. It's an assault course. The waiting room has a television playing a show you can't follow, a crying child, fluorescent lighting with a hum you can hear, a seat whose texture you can feel through your clothes, antiseptic layered over the next patient's perfume. Then you're called in, and the doctor — who may be perfectly lovely — asks questions that require you to access your internal experience in real time, under pressure, on paper that crinkles with every movement. The sensory load alone is enough to make you avoid it; add the social load — the performance of being a normal patient, the fear of being disbelieved — and avoidance stops being a choice

and becomes a survival response. You're not avoiding the doctor. You're avoiding the gauntlet that sits between you and the doctor.

Interoception. Interoception is your brain's ability to read the internal state of your body — hunger, thirst, pain, the early warning signs of illness. Autistic and AuDHD people often have atypical interoception: for many of us the signals are either very quiet or very loud, with no middle ground. If yours arrive quietly, you don't just forget to eat — you don't notice you're hungry until you're shaking, and you don't notice you're getting ill until you're properly ill, because the gradual ramp of symptoms never crossed your awareness threshold. When you do feel something, it's often only at the extreme end — which is why you might end up in A&E with something that's been building for weeks. You're not ignoring your body. Its announcements go to a mailbox your brain checks rarely, and only when the parcel is too big to miss.

The masking habit of pushing through. Finally, there's the habit you learned before you knew you had either condition: pushing through. As a girl, you learned that your discomfort — sensory, social, physical — was invisible to other people, and that complaining got you labelled difficult, dramatic or lazy. So you built a lifetime of ignoring your body's protests and continuing anyway. You finished the school day with a migraine. You went to work with a fever. You smiled through the event while every sense screamed. This is an extraordinary skill, and a disaster for your health: pushing through trains you not

to notice — and sometimes not to have — the early signals that would tell you to slow down. By the time a signal is undeniable, the problem has usually grown. You've been so good at ignoring your body that it has learned to be ignored.

What It Costs

The neglect has a price, and it's not the kind of price you can pay quietly. The first cost is burnout. Your body is the infrastructure of your life — the thing every other thing runs on. When it runs on fumes — skipped meals, poor sleep, deferred care — everything else degrades with it: the emotional regulation that was already stretched thin collapses, the sensory tolerance that was already low drops further, the executive function that was already unreliable gets worse. Health neglect doesn't just make you unwell; it makes your AuDHD harder to live with in every direction. You can't out-organise a body running on empty, or out-think a brain short on fuel.

The second cost is that small problems become big ones. The symptom you ignore for six months becomes the diagnosis that requires surgery; the appointment you postpone for two years becomes the treatment that requires three. Deferred care is compounding interest in the worst sense: the longer you wait, the more you owe, and the more daunting the debt feels, which makes you defer further. It's a spiral that tightens with every "I'll deal with it later."

The third cost is the one that hurts most: the gaslighting. When you finally show up — exhausted, with a body that's been sending signals you couldn't read and complaints you couldn't articulate — the system is often not ready for you. You explain your symptoms the way your AuDHD brain actually experiences them: scattered, compressed, minimised, because that's what you've trained yourself to do. The doctor hears a vague, anxious woman who doesn't seem to know what's wrong. Maybe they tell you it's stress. Maybe they tell you to lose weight. Maybe they run one test, find nothing, and close the file. You leave feeling like you imagined it — and you do what you've always done, decide your body was lying, and push through. The medical system gaslights neurodivergent women as a matter of routine, not malice: it's built for people who can describe symptoms in order, remember their history, tolerate waiting rooms and advocate under fluorescent lights. That's a double injury — the original problem, and then blame for not presenting it properly.

The Reframe

So here's the reframe, and I want you to actually take it in, because it changes everything downstream. Your health neglect is not a moral failure. It is a system problem. Your brain was not designed to manage a human body — at least, not the way the world expects. The tasks involved are exactly the tasks you're worst at:

remembering, planning, initiating, persisting, reading internal signals, enduring hostile environments. When the system fails, the correct response is not more guilt — it's a better system. Guilt has had decades to try, and it has not fixed a single tooth, refilled a single prescription, or made a single appointment. It is the most expensive and least effective health intervention ever invented, and you've been self-administering it your whole life.

The good news — and it is genuinely good — is that system problems have system solutions. You don't need to become a different person; you need different tools: an external brain for the appointments you can't remember, alarms for the time you can't feel, food that doesn't require cooking, a doctor's letter written in advance so you don't have to perform in the moment. None of it requires willpower. It requires setup — and setup, unlike character, can be done in an afternoon and improved forever after.

Every chapter in this book is a system built on this foundation. Sleep, food, movement, hormones, medical appointments, medication — each one gets the same treatment: understand the mechanism, remove the shame, install a system that works with your brain instead of against it. You are not broken. You have a brain with a specific architecture, living in a world and a body that were both designed for someone else. The rest of this book is about closing that gap — not by changing who you are, but by changing what you run on.

Start with one thing, and let it be small. Not the dentist. Not the smear test. Not the five-year health overhaul. One thing: put a glass of water by your bed and drink it when you wake up. Or set a daily alarm called "eat something." Or write the dental appointment request as a text message to yourself, so the task lives somewhere you can actually see it. Small systems compound exactly the way neglect does — except they compound upward instead of down. Your body is the infrastructure of your life. It's been running on neglect for long enough. It's time to give it the same care you give everything else — not because you're failing, but because you deserve better infrastructure.

Toolkit Summary

- Health neglect in AuDHD women is patterned, not random: forgetting to eat, postponing appointments, ignoring symptoms, and "I'll deal with it later" as a way of life.
- Every health task is an executive function task, and executive function is exactly what the AuDHD brain struggles with — "just do it" advice assumes a chain of steps that doesn't exist for you.
- Four mechanisms drive the neglect: executive dysfunction, time blindness, sensory avoidance of medical environments, and atypical interoception (quiet body signals).

- The masking habit of pushing through trains you to ignore your body's early warnings, so problems arrive as crises.
- The costs compound: burnout, small problems becoming big ones, and medical gaslighting when you show up unable to perform the role of the "good patient."
- The reframe: this is a system problem, not a moral one. Guilt has never fixed anything; systems can. Start small and let the systems compound upward.

Chapter 2: The Executive Function of Healthcare

HERE IS A LIST OF THINGS THAT HAPPENED TO YOU LAST YEAR, IN NO PARTICULAR ORDER. YOU MISSED A DENTAL APPOINTMENT BECAUSE YOU WROTE THE DATE DOWN SOMEWHERE YOU CAN NO LONGER FIND. YOU RAN OUT OF YOUR MEDICATION ON A FRIDAY AND SPENT THE WEEKEND UNMEDICATED BECAUSE THE REPEAT PRESCRIPTION REQUEST NEEDED TO BE SUBMITTED BEFORE MIDDAY AND YOU SUBMITTED IT AT 12:07. YOU RECEIVED A LETTER FROM THE HOSPITAL SAYING YOU'D BEEN DISCHARGED FROM A WAITING LIST YOU DIDN'T KNOW YOU WERE ON, FOR A REFERRAL YOU DON'T REMEMBER BEING MADE. YOU FOUND A FORM FOR A BLOOD TEST TUCKED INTO A PILE OF POST — BY THEN IT WAS THREE MONTHS OUT OF DATE. YOU SPENT AN HOUR ON HOLD TO THE GP SURGERY, GOT THROUGH, AND THEN FORGOT WHAT YOU WERE CALLING ABOUT WHILE YOU WERE WAITING, HUNG UP, AND NEVER CALLED BACK. YOU ARE NOT DISORGANISED. YOU ARE RUNNING A HUMAN BODY ON A BRAIN WHOSE EXECUTIVE FUNCTION — THE SYSTEM THAT PLANS, INITIATES, REMEMBERS AND FOLLOWS THROUGH — WAS NEVER INSTALLED IN FULL.

Here is the sentence that this whole chapter is built on, so let's say it plainly: healthcare is not one thing you occasionally do. Healthcare is an enormous pile of small executive function tasks, and someone with AuDHD is

running that pile on the exact hardware it was designed to break. The previous chapter showed you why you neglect your health. This one is about what happens when you actually try to engage with it — because engaging with the health system is itself a series of traps, and knowing where the traps are is the first step to building a bridge over them.

Healthcare Is a Task Stack

Let's break the health system down into what it actually is, from your side of the desk. It is not "going to the doctor." It is a stack of tasks, each with its own failure modes, and it sits on top of you permanently whether you're engaged with it or not.

Remembering appointments. The appointment itself is the easy part. Everything around it is not. You have to remember the appointment exists (prospective memory — remembering to remember), know what day it's on (time blindness — it will ambush you anyway), get there on time (transitioning between contexts, which your brain treats as a tax), and arrive prepared (which requires having thought about it before the waiting room). A missed appointment isn't usually carelessness. It's a failure at any one of four separate points in a chain, and the chain is invisible until it snaps.

Refilling prescriptions. This is a task that requires you to notice a future problem (I'm running out), act on it in advance (before the empty bottle is the evidence), and

do it through whatever interface your system uses — a phone line open nine-to-four, an app that may or may not work, a form you have to print. For a brain that struggles with the future as a concept and forms as a genre, refilling a prescription on time is a small miracle, and you're expected to perform it monthly, forever. Then, when you finally get the medication, you have to actually take it — daily, at the same time, for weeks or years — which is a persistence task, the single hardest category for an ADHD brain.

Following treatment plans. Here's a cruel irony: the more seriously ill you are, the more executive function the treatment demands of you. Physiotherapy is a daily homework assignment. Diabetes management is a full-time administrative job. Antibiotics require a twice-daily alarm for a week. The people who design treatment plans assume a baseline of organisational competence that you don't have, and then they blame you — or you blame yourself — when the plan collapses. If a treatment plan doesn't survive contact with your actual life, that's a design flaw in the plan, not a character flaw in you. But nobody tells you that, so you carry the failure privately, and the next time you need treatment you're carrying shame into the room with you.

Tracking symptoms. Medical care depends on information you're structurally bad at producing. You're asked to describe your pain on a scale, remember when it started, notice patterns over weeks, and report what makes it better or worse. For a woman whose

interoception is quiet, whose memory for the past is a fog, and whose instinct under questioning is to minimise and perform normality, symptom reporting is a high-stakes exam you've never studied for. You're not withholding information. You genuinely don't have it — because nobody built you a system for collecting it in real time, and your brain doesn't do retrospective data collection.

The Healthcare Admin Pile

Underneath all of this sits the thing that never gets discussed in health advice, because it's not medical and it's not interesting: the admin. The mail. The forms. The insurance. The letters that say "please complete the attached questionnaire." The emails about appointments you need to confirm by clicking a link you'll lose. The prescription charges, the referrals, the test results that arrive as cryptic clinical language you have to decode, the follow-up you're supposed to request but the letter doesn't say so explicitly.

Most people process this stuff the way they process other mail: they open it, they handle it, they're done. You don't — because opening mail is itself a task with multiple steps, and because the pile is where executive function goes to die. The pile isn't just a pile of paper. It's a pile of unresolved tasks, each one generating a low-level hum of anxiety every time you walk past it, and each one requiring a decision you keep deferring. The pile grows, and the shame about the pile grows with it, and

eventually the pile becomes a single terrifying object that you can't look at, which means the one letter in it that actually mattered — the appointment, the test result, the deadline — gets buried under the weight of the whole.

Here's what you need to understand about the pile, and about all of healthcare admin: it is not a moral test. It is an unmanaged system. Mail is an input stream with no processing pipeline. Forms are tasks with no trigger. Insurance is a project with no owner. The pile exists because nobody ever built you the pipeline — and the solution is not to try harder, it's to build the pipeline.

The External Brain

The single most useful concept in AuDHD healthcare is the external brain: a system outside your skull that does the remembering, reminding, and tracking that your working memory can't be trusted with. Your internal brain is not going to get better at this — stop waiting for that upgrade. The external brain is the upgrade.

The external brain has three components, and you get to choose the ones that work for you. **Apps and digital tools** are the first. Your phone is genuinely good at being an external brain: it can hold appointments, send alarms, remind you to take medication, and nag you to eat. The failure mode is that apps themselves become a pile — you download eleven, use two for a week, and abandon them. The rule is one tool per job, chosen once, not the perfect tool. If you can't remember to use an app, the app is the

wrong app — the right one is the one that pushes itself into your face. **Alarms** are the second component, and they're more powerful than they look, because they convert a prospective memory task ("I must remember to call the pharmacy") into a triggered task ("the phone is ringing and the note says 'pharmacy'"). You don't need to remember; you just need to obey the ring. **Visible calendars** are the third. A calendar you can see — on the wall, on the fridge, on the home screen of your phone — is a calendar that does time-blindness correction work every time you glance at it. The invisible calendar in your phone that you never open is not a calendar; it's a storage unit. Visibility is the whole point.

The external brain only works if it's honest, so one rule matters more than any other: if it's not in the external brain, it doesn't exist. Stop trusting your memory for anything health-related. The moment you think "I'll remember that", write it down, set the alarm, put it on the wall. Your memory is not part of the system. The system is the system.

Body Doubling for Medical Admin

Here's a strategy that sounds odd and works unreasonably well: don't do medical admin alone. Body doubling — having another person present while you do a task — works because it outsources your initiation and persistence problems to someone else's presence. You

don't need them to help. You need them to be there, so the task becomes a shared social event instead of a private mountain.

The applications are endless. Have a friend sit in the room (or on a video call) while you make the phone call you've been avoiding for a month — you can even put them on speaker, which has the side benefit of a witness for anything the receptionist says. Have your partner or a housemate sit with you while you open the pile, one envelope at a time, sorting into "handle now" and "shred" and "file." Some people use paid body doubles or focus apps that connect you with strangers for exactly this purpose — it sounds like a scam, and it is not; it's using the social brain to run the administrative brain, which is what your social brain is for. The only rule is that the body double's job is presence, not doing it for you. They're the scaffolding, not the builder. You're the builder.

The One-Touch Rule

The pile grows because mail passes through your hands without being processed. The antidote is the one-touch rule, and it's brutally simple: every piece of health paperwork gets touched exactly once, and at that moment, it goes to its final destination. Not the pile. Not "I'll deal with this later." Its final destination.

What are the final destinations? There are only four, and you can write them on a sticky note above wherever mail lands. **Do it now** — if it takes under two minutes

(sign the form, put the appointment in the calendar, set the reminder alarm), do it at the moment you touch it. **Put it in the action tray** — if it needs a phone call or a decision, it goes into a single, visible tray, and the tray gets a weekly appointment in your calendar called "admin hour", and at that appointment the tray is processed to zero. **File it** — if it's information only (a test result you need for reference, a letter confirming something), it goes into a folder labelled with the year, and the folder lives in one place, and the place is the same place every time. **Shred it** — if it's junk or outdated, it dies immediately. Four destinations. One touch. That's the whole system.

The one-touch rule works for AuDHD brains because it removes the two failure points that make the pile grow: the decision ("what do I do with this?") and the deferral ("I'll do it later"). The decision is pre-made by the four categories, and "later" has been replaced by the action tray, which has a scheduled appointment with reality. You will still miss things — the rule is not perfection, it's compression. A pile that gets processed weekly is a system. A pile that never gets touched is a monument.

Simplifying Your Health System

Finally, the deepest fix: make the system itself smaller. Most people's healthcare involves an unnecessarily large number of moving parts, and every moving part is

another executive function task. You can shrink the system, and shrinking the system is a legitimate medical intervention — not a cop-out, an optimisation.

Here's what simplification looks like in practice. **One pharmacy**, and the same one every time, so refills become a single familiar interaction instead of a fresh negotiation. **One GP**, seen consistently, so you don't re-tell your history to a stranger every visit — and if your practice doesn't allow continuity, that's a reason to consider switching, because continuity is medically valuable for neurodivergent patients, not a luxury. **Repeat prescriptions on automatic reorder** wherever the system allows it, so the monthly miracle stops being required of you. **Standing appointments** — book the next one while you're in the room, every time, so nothing depends on future-you making a call. **One notebook or notes app for all health information** — symptoms, questions, medication lists, test results — so you never have to assemble your own medical history from memory in a waiting room. **Pre-written appointment scripts** — a note in your phone that says what you're there for, what you need to ask, and what outcome you need, so the sensory-social pressure of the room doesn't erase your ability to speak.

Simplification is also about removing shame from the system. The goal of all of this is not to become a person who manages healthcare flawlessly. The goal is to become a person whose healthcare system requires as little executive function as possible — because your executive

function is a finite resource, and it has better things to do than fight with the pile. Every hour you don't spend wrestling the admin pile is an hour of your actual life. That's what you're building here. Not a perfect patient. A system that runs, so you can.

Toolkit Summary

- Healthcare is a stack of executive function tasks — remembering appointments, refilling prescriptions, following treatment plans, tracking symptoms — each with its own failure mode for an AuDHD brain.
- The healthcare admin pile (mail, forms, insurance) is an unmanaged input stream, not a moral test. It grows because there's no processing pipeline.
- Build an external brain: one app per job, alarms that convert "remember to" into "do it now", and a calendar you can actually see. If it's not in the external brain, it doesn't exist.
- Body doubling turns medical admin into a shared task — have someone present while you make the call or open the pile. Presence is the scaffolding, not the doing.

- Use the one-touch rule for paperwork: do it now, action tray (processed at a weekly admin hour), file it, or shred it. One touch, four destinations, no pile.
- Simplify the whole system: one pharmacy, one GP, automatic reorders, standing appointments, one health notebook, pre-written appointment scripts. Fewer moving parts means less executive function required.

Chapter 3: Sleep: The Foundation Everything Sits On

IT'S 11:47PM AND YOU ARE EXHAUSTED — THE HOLLOW EXHAUSTION OF A DAY THAT DEMANDED PERFORMANCE FROM THE MOMENT YOU WOKE UP. YOUR BODY IS HEAVY, YOUR EYES BURN, YOU SHOULD BE ASLEEP. INSTEAD YOU ARE LYING IN THE DARK WHILE YOUR BRAIN PRESENTS A COMPREHENSIVE DOCUMENTARY OF EVERY EMBARRASSING THING YOU'VE EVER SAID, IN CHRONOLOGICAL ORDER, WITH COMMENTARY. THEN TOMORROW'S TO-DO LIST, RECITED IN A LOOP. THEN SOMETHING YOU NEED TO BUY. THEN THE DOCUMENTARY AGAIN, WITH A NEW EPISODE. YOUR BODY IS BEGGING FOR REST; YOUR BRAIN HAS DECIDED THAT MIDNIGHT IS THE PERFECT TIME TO PROCESS THE ENTIRE CONTENTS OF YOUR LIFE. THIS IS NOT A BAD NIGHT. THIS IS MOST NIGHTS.

If this is you, you are not doing sleep wrong. You are trying to sleep with an AuDHD brain, which is a different activity entirely. Sleep sits at the intersection of everything this book is about: sensory processing, executive function, emotional regulation, and the war between the half of you that needs rest and the half that finds rest under-stimulating. This chapter is about understanding that war — and winning it with a system, not discipline.

Why Sleep Is Different for Your Brain

Let's start with the things that actually happen at night, because they're not random and they're not your fault. The first is the racing brain. During the day, your thoughts are moving at their normal AuDHD pace, but they're competing with the world — there's input, there are tasks, there's noise to drown them out. At night, the input stops. The world goes quiet. And your thoughts, suddenly the only show in town, seize the stage. What you experience as "my brain won't shut up" is really your brain finally having your undivided attention and making the most of it. Racing thoughts at bedtime aren't a sign that you're anxious (though anxiety can make them worse). They're a sign that your brain never learned to wind down, because it was never given a wind-down protocol — and nobody winds down by accident.

The second is sensory. Sleep is a sensory activity, and your sensory system is not a neurotypical one. The label in your pyjamas. The seam of the sheet. The sound of the neighbour's boiler. The temperature of the room being half a degree off. The light from the router blinking through the gap in the curtains. For most people, these are background details their brains filter out. For you, every one of them is a full-volume announcement, and your body cannot sleep while it's being announced at. You're not being fussy about the sheets. You're trying to sleep in an environment that is attacking you, and your body is responding appropriately.

The third is your body clock. Many AuDHD people — and a striking number of autistic people — have delayed sleep phase: your natural sleep window is shifted later than the world's. Your body wants sleep at two in the morning and wake at ten. This isn't a preference; it's chronobiology, as real as being left-handed. The world's schedule runs against it, leaving you in permanent social jetlag — asleep at the wrong time, awake at the wrong time, exhausted in between, the exhaustion invisible because you're performing so well.

And the fourth has a wonderful, terrible name: revenge bedtime procrastination — the refusal to sleep, not because you're not tired, but because sleep feels like the end of the day, and the day was not yours. It belonged to work, to other people, to masks and obligations. Sleep would be the final surrender of the last scrap of time that belonged to you. So you stay up — not because the content is good, but because staying up is the only rebellion available, and the only place you feel like yourself. The revenge is real, and the debt comes due every morning.

Why Sleep Matters More for You

Here's where the stakes get serious, because sleep is not just a nice thing that happens to well-organised people. Sleep is the foundation that everything else in this

book sits on — and for an AuDHD woman, the foundation matters more, because sleep deprivation hits your specific weaknesses hardest.

Emotional regulation. Your emotional system runs fast and hot at the best of times; sleep deprivation removes the last insulation. The rejection sensitivity that was manageable becomes unbearable; the flooding that was occasional becomes constant. Research is unambiguous: a tired brain over-reacts to negatives and under-registers positives. For a brain that already runs on a hair trigger, sleep loss isn't tiredness — it's emotional instability with a timer.

Executive function. Executive function runs on sleep the way a phone runs on battery. A sleep-deprived AuDHD brain isn't a tired version of your brain — it's your brain with the management team on strike. Working memory shrinks, initiation collapses, impulse control evaporates. Everything this book teaches assumes a brain that can run it, and a sleep-deprived brain can't run anything. Sleep isn't a luxury on top of the systems — it's the operating system they run on.

Sensory tolerance. And then the cruelest feedback loop: poor sleep lowers sensory tolerance — everything louder, brighter, scratchier — which means more overload, which means more exhaustion, which means worse sleep. But the loop runs both ways: improving sleep improves sensory tolerance, which reduces overload, which improves sleep. Sleep is your highest-leverage intervention, because it feeds every other system at once.

The Wind-Down Protocol

The first fix is the one nobody gives you: you need an actual wind-down protocol, because your brain will not wind down on its own. It needs a ramp, not a cliff. The neurotypical brain can read a chapter, put the book down, and be asleep in twenty minutes. Your brain needs an hour of deliberate, boring, sensory-soothing transition before it will even consider it. This is not a failure; it's an engineering requirement.

The protocol has three parts, adaptable to your actual life. **The shutdown:** a fixed time — an hour before you want to be asleep — when stimulating input stops: no scrolling, no work, no high-dopamine anything. This is the hard part, because it collides with revenge bedtime procrastination, so be honest: the phone is not rest, it's input, and input is what's keeping you awake. **The settle:** twenty to forty minutes of low-stimulation, low-dopamine activity that genuinely soothes you — reading something deliberately boring (not the thriller), a warm shower, stretching, folding laundry, the same album you've heard a hundred times. The rule is sameness: roughly the same wind-down every night, because routine tells the autistic half of your brain the day is over and it's safe to stop processing. **The handover:** lights down, screens off, into bed at the same time, so your body learns the ritual means sleep.

The Sensory Sleep Environment

The second fix is the environment, and this is where your autistic half gets to be as particular as it likes. The perfect sleep environment for a neurotypical person is a vaguely dark, vaguely quiet room. The perfect sleep environment for you is a sensory fortress, and building one is a legitimate use of your attention to detail.

Work through the senses, one at a time, and fix what you find. **Light:** blackout curtains or an eye mask — not as a luxury, but as a necessity, because your brain will process every photon it can find. **Sound:** whatever the opposite of "annoying" is for you — earplugs, or white/brown/pink noise, or the same audiobook every night (sameness again — the brain stops tracking it and drifts). **Touch:** sheets and pyjamas that don't bother you, washed in the same detergent, and no shame about the fact that you own exactly one set of sheets that are acceptable — buy a second set of the identical ones and rotate. **Temperature:** cooler than you think — the body needs to drop temperature to sleep, and a warm room fights that. **Weight:** if you like deep pressure, a heavy blanket is not a gimmick; it's sensory regulation you can sleep in. The point is not to achieve a perfect room on the first night. The point is to treat every sleep annoyance as a fixable problem rather than a personal failing — because it is.

The 10-3-2-1-0 Rule, Adapted

You may have heard of the sleep-hygiene rule that goes: ten hours before bed, no caffeine; three hours before, no food or alcohol; two hours before, no work; one hour before, no screens; zero times you hit snooze. It's a good rule — for someone with a functioning working memory and a normal relationship with time. You need the adapted version, because the original assumes you can remember the rules at the moment they matter, and time-blindness means you will drink the coffee at four, remember the rule at ten, and then be cross with yourself at midnight.

The adapted version externalises the rules. **Set the alarms:** a "last coffee" alarm, a "start winding down" alarm, a "get into bed" alarm, each named so future-you knows what it's for. You're not remembering the schedule; the schedule is reminding you. **Do the caffeine cut-off in the morning:** decide once that you don't drink caffeine after 2pm, and write it on a sticky note by the kettle, so you never make the decision under time-blindness. **Move the screen rule earlier:** the no-screens-before-bed rule collides with revenge bedtime procrastination and loses, so instead of fighting it, build screens into the wind-down — an e-reader with warm light, or the same familiar show at low volume. Scrolling is input; familiar comfort viewing is sameness. They feel identical and are not. And **snooze:** either embrace it (set

the alarm for your real wake time) or ban it (put the alarm across the room), but decide once — you should not be negotiating with yourself at 6:47am before coffee.

Handling the Racing Brain

And then there's the night itself — the 3am brain that has decided to process everything. First, know that fighting the racing brain makes it race harder; the more you try to suppress the thoughts, the more they demand attention. The strategies that work are the ones that give the thoughts somewhere to go.

The brain dump. Keep paper and a pen by the bed. When the thoughts start, write them down — not neatly, just out. The to-do list, the embarrassment, the thing you should have said: all of it. This works because the thoughts are often your brain holding onto information it's afraid of losing; once it's on paper, your brain can let it go. If the thought is "I must not forget this", the paper is the external brain, and your actual brain can clock off.

The boring tape. Have one pre-prepared, extremely boring mental tape for when the thoughts are not about remembering but about processing. The classic is counting breaths, but "counting breaths" is too interesting for some brains, so pick something blander: recite the alphabet backwards slowly, name the countries of the world in alphabetical order, mentally walk through

your own kitchen describing every object in it. The goal is not mindfulness. The goal is to be so boring that your brain gives up and goes to sleep out of sheer disinterest.

The get-up rule. If you've been lying in the dark for more than about twenty minutes and sleep is not coming, get up. Go to a dim room, do the most boring thing you own (fold socks, read a manual), and return to bed when you're sleepy again. This sounds counterintuitive, but lying in bed awake trains your brain to associate bed with wakefulness and anxiety — and that association is the enemy. You want bed to mean sleep. If bed means struggle, get out of bed and let the struggle happen somewhere else.

When to Seek Help

Finally, the part nobody puts in the sleep chapter, because it's uncomfortable: some sleep problems are medical problems, and AuDHD women are disproportionately likely to have them. If you have tried the environment and the wind-down and the rules and you are still exhausted — especially if you wake up exhausted after a full night, or you snore, or you wake gasping, or your partner says you stop breathing — you need to talk to a doctor about sleep-disordered breathing. Sleep apnoea is common in people with ADHD, it is badly underdiagnosed in women (who often present with

fatigue and morning headaches rather than the classic loud snoring), and it is treatable. Treating it can change your life more than any sleep-hygiene rule ever could.

Also worth discussing with a professional: a stubbornly delayed sleep window that's wrecking your life, racing thoughts no strategy touches, or bad sleep alongside depression or anxiety. And if your sleep is affected by ADHD medication timing — stimulants late in the day push sleep later, and some people need the timing adjusted — that's a conversation for your prescriber. Sleep is not a moral achievement. It's a medical and sensory system, and when it's broken, the fix is help, not willpower.

Here's the summary in one sentence: your brain is not built to fall asleep by accident, so stop expecting it to, and build the ramp instead. The wind-down, the fortress, the alarms, the boring tape — these aren't rules to be good at; they're scaffolding for a brain that needs scaffolding. Sleep is the foundation everything else sits on. Build it, and everything in this book gets easier; fight it, and everything gets harder. You get to choose — and you're allowed to choose the ramp.

Toolkit Summary

- Sleep is different for the AuDHD brain: racing thoughts at night, sensory intolerance in bed, delayed sleep phase (a shifted body clock), and revenge bedtime procrastination (staying up because the day wasn't yours).
- Sleep matters more for you because it directly powers emotional regulation, executive function, and sensory tolerance — a tired AuDHD brain is an unstable, non-functional, overload-prone one.
- Build a wind-down protocol: a shutdown (no input), a settle (twenty to forty minutes of deliberately boring sameness), and a handover (same time, lights down) — your brain needs a ramp, not a cliff.
- Make the bedroom a sensory fortress: blackout, sound, acceptable sheets, cool temperature, weight if you like pressure. Treat every annoyance as a fixable problem, not a failing.
- Adapt the 10-3-2-1-0 rule with externalisation: set named alarms, decide caffeine cut-offs once, and stop negotiating with yourself at midnight.
- Handle the racing brain with a brain dump (paper by the bed), a deliberately boring mental tape, and the get-up rule — don't train your bed to mean struggle.

- Seek help for the medical layer: sleep apnoea is common and underdiagnosed in women with ADHD, and medication timing can affect sleep. Sleep is a system, not a moral achievement.

Chapter 4: Food, Eating, and the AuDHD Brain

IT IS 4:37PM, AND YOU HAVE JUST REALISED YOU ARE STARVING. NOT HUNGRY — STARVING, IN THE HOLLOW, SHAKY, SLIGHTLY NAUSEATED WAY THAT MAKES YOUR HANDS TREMBLE AND YOUR THOUGHTS GO STATIC. YOU TRY TO REMEMBER WHAT YOU'VE EATEN TODAY. COFFEE. HALF A CROISSANT AT 9AM WHILE STANDING UP. A HANDFUL OF CRISPS AT LUNCHTIME THAT YOU ATE OVER THE SINK WHILE WAITING FOR THE KETTLE, WHICH YOU THEN FORGOT ABOUT, SO THERE'S A COLD CUP OF TEA SOMEWHERE THAT YOU WILL FIND TOMORROW. AND NOW IT'S 4:37 AND YOUR BODY IS STAGING A PROTEST, AND THE ONLY THING YOU CAN THINK ABOUT IS SUGAR — SPECIFICALLY, THE CHOCOLATE BAR IN THE DRAWER, WHICH YOU ARE GOING TO EAT, AND WHICH IS GOING TO SPIKE YOUR BLOOD SUGAR, AND WHICH IS GOING TO CRASH YOU INTO THE SOFA IN ABOUT FORTY MINUTES, WHERE YOU WILL SCROLL FOR TWO HOURS AND EAT NOTHING, AND THEN YOU'LL BE HUNGRY AGAIN AT 9PM AND EAT SOMETHING ELSE FROM THE DRAWER, AND THEN YOU'LL BE AWAKE AT MIDNIGHT WITH A RACING HEART AND A STOMACH THAT REGRETS EVERYTHING.

You know this cycle. You've lived it a thousand times. You have probably been told — by doctors, by articles, by well-meaning people — that the answer is meal prep, or meal planning, or "just eat three balanced meals a day."

And you have tried. You have bought the meal-prep containers. And somewhere between the containers and the reality of cooking after a day of being a person, the plan collapsed, and you added "can't even feed myself properly" to the list of things you're ashamed of.

Here's the thing you were never told: the food cycle you're living is not a discipline problem. It's an AuDHD food system — a predictable, mechanical result of the way your brain handles hunger, dopamine, sensory input and executive function. And like every other system in this book, it can be redesigned. Not by becoming a different person who loves cooking and meal-preps on Sundays. By building a food system that works with the brain you actually have.

The AuDHD Food Cycle, Named

Let's name the parts, because each one has a mechanism, and mechanisms can be worked with.

Forgetting to eat. Your hunger signals are unreliable — quiet, late, or absent entirely until the emergency stage. Add time blindness and hyperfocus, and eating becomes something that simply doesn't occur to you. This is not a metaphor: you can be so absorbed in a task that your brain genuinely does not register the passage of hours or the body's requests. The food is not forgotten because you don't care about food. It's forgotten because eating is a future-oriented, self-monitoring task, and your brain is busy and your body's announcements are quiet.

Hyperfocus without food. The flip side of forgetting: when you're in hyperfocus — that glorious, locked-in state where the work flows and the world disappears — food is an interruption. Stopping to eat breaks the state, and your brain, which values the state above almost everything, will sabotage the meal. You tell yourself "I'll eat when I finish this." "This" is never finished. Hyperfocus is a resource that eats through everything else, including meals, and the crash at the end — the shaking, the fog, the irritability — is the price.

Sensory food aversions. You have a list of foods that are simply not acceptable. Mushy textures. The sound of chewing certain things. Food that touches other food on the plate. Food with a smell that arrives before the taste does. This isn't fussiness; it's a sensory system that processes food at full volume, and it means your edible-world is smaller than most people's — which is fine, and normal for us, and completely at odds with every dietary guideline ever written. The guidelines assume a person who can eat anything. You can't, and the gap between what you're "supposed" to eat and what you can actually eat is a daily source of shame that serves no one.

Dopamine-seeking snacks. And then there's the snacking. Your brain runs on a dopamine deficit at baseline, and food — sugar, salt, fat, crunch — is a reliable, legal, instant source. The biscuit isn't hunger; it's your brain requesting a chemical. That's why the chocolate bar in the drawer has so much power — not willpower weakness, but neurochemistry, the same

neurochemistry that drives the whole cycle. The snack gives the hit, the hit crashes, the crash reaches for the next snack, and the cycle self-perpetuates — especially at the low-dopamine times of day (the afternoon slump, the evening wind-down), which are exactly when you'll find yourself in the kitchen without having decided to go there.

What It Costs

The cycle has consequences, and they're not just "you ate a biscuit." The first is the energy crash: the blood-sugar rollercoaster that comes from eating little and then eating sugar. Your energy doesn't just dip — it plummets, and the plummet lands on executive function, mood and sensory tolerance all at once. The 4pm crash is not a personal weakness. It's what happens to a body that's been running on fumes and then gets hit with a sugar spike; the spike is followed by the insulin surge, and the insulin surge is followed by the crash that sends you to the sofa.

The second is mood destabilisation. Your emotional system already runs fast and hot; add blood-sugar swings — the irritability of the empty stomach, the anxious buzz of the spike, the flat despair of the crash — and you're not just hungry, you're emotionally unstable, in a way that looks like a mood problem and is actually a food-timing

problem. So many AuDHD women are treated for mood disorders when what they need is a sandwich and a system.

And the third cost is the shame, which is the heaviest of all. You have internalised the message that feeding yourself is basic, that everyone does it, that the fact that you can't is proof of something wrong with you. The shame doesn't help you eat better — it makes you hide the way you actually eat, avoid talking about it, and eat worse in private. Shame is the fifth food group in the AuDHD diet, and it's the only one with no nutritional value at all.

The Eat-on-Autopilot System

The redesign starts with one principle, and I want you to tattoo it somewhere: **the goal is not a perfect diet. The goal is to eat — consistently, without requiring executive function or willpower.** A system where you eat a tolerable amount of acceptable food most days will outperform a perfect plan you can't run. Every strategy in this chapter is in service of that principle.

The core of the system is autopilot food: food that requires no decisions and almost no steps. Decisions are the enemy — every "what should I eat?" is an executive function task, and by the time you're hungry enough to ask, you're too depleted to answer well. Autopilot food removes the question. Build a short list of foods that meet three criteria: you can eat them (sensory-acceptable),

they're always available (bought on repeat, visible in the cupboard), and they require zero or near-zero preparation (open, pour, microwave, eat). For most of us this looks like: yoghurt drinks or meal-replacement shakes, bread with something on it, ready-made soup, cheese, fruit that doesn't need cutting, nuts, boiled eggs, instant porridge, tinned beans, protein bars. The list is personal and allowed to be boring — the same five foods every day is not a failure of imagination, it's a working system. Sameness isn't just acceptable; it's the point.

Snacks that count. Here's the upgrade that changes everything: make your snacks do the work of meals. A snack that contains protein and fat will hold you for hours; a snack that's pure sugar will crash you in forty minutes. The dopamine-seeking snack is not going away — don't fight it, route it. Pair the dopamine food with a protein food: the chocolate bar plus a handful of nuts, the crisps plus a cheese string, the biscuit plus a yoghurt. The combination satisfies the dopamine request and keeps your blood sugar level enough that the crash doesn't land. You're not banning the snack. You're upgrading its architecture.

The dopamine-free protein trick. And a specific trick for the times when you know you need to eat but food holds no appeal: protein first, without decision or ceremony. A glass of milk. A pre-made protein shake (kept in the fridge, drunk like a drink, not a meal). A slice of ham eaten straight from the packet. This is not a meal and it doesn't need to be one. The goal is to get a protein

baseline into your body so the blood sugar doesn't swing and the crash doesn't land, so that later — when you're less depleted — you can eat an actual meal without the desperation. The trick works because it removes every barrier: no cooking, no plate, no chewing if you don't want to chew, no decision. It's food as medication, taken on a schedule, and it works.

Sensory-Friendly Food Lists

Because your sensory system has a vote, give it a say in the design. Make your actual list — the foods acceptable to you right now — and treat it as a legitimate document, not a confession. Write down the safe foods, the safe textures, the safe temperatures (some of us can only eat things that are room temperature, or hot, or cold — a preference, not a problem). This list is your edible world, and the system gets built entirely inside it.

Then optimise within it. Find the version of each safe food that requires the least work — the pre-cut fruit, the ready-cooked chicken, the tinned version, the frozen version. Accept that the "proper" version (whole, raw, from scratch) is not the goal. A sensory-acceptable food that actually gets eaten beats a nutritionally-perfect food that stays in the fridge. And rotate the list slowly — one new acceptable food every few weeks, tried in a low-pressure context, no forcing — so your world slowly grows instead of shrinking. The list is not a cage. It's a foundation.

Meal Prep for Executive Dysfunction

Meal prep as it's normally taught — the Sunday-afternoon, batch-cook, twelve-containers ritual — is an executive function project with a deadline, a perfect candidate for AuDHD failure. So redesign it. The principle: **prep in the moment of capability, not on a schedule.** When cooking feels possible — a hyperfocus day, a weekend morning with energy — that's when you prep, with a specific, limited set of moves.

The moves: boil eggs (they keep for a week and need zero thought). Cook rice or pasta in bulk and freeze in single portions (three minutes in the microwave). Buy pre-prepped instead of whole — it costs more and is worth every penny, because your executive function is worth more than the difference. Keep a drawer of "always fine" foods — shelf-stable, sensory-acceptable, no-cook — for the days cooking isn't happening. And accept the frozen aisle as a legitimate source of meals: frozen vegetables, rice and protein were harvested at their peak, require zero prep, and are nutritionally fine. The frozen aisle isn't a fallback. It's a system.

When Eating Disorders Are Involved

And now the part that needs to be said, carefully and without euphemism: for some of us, the relationship with food goes beyond the cycle into disordered eating, and the AuDHD and autistic brain makes us more vulnerable

to it. The same mechanisms in this chapter — the need for control, the sensory intensity, the dopamine chasing, the black-and-white thinking — can turn into restriction, bingeing, purging, or a fixation on food rules and body shape that no food system can fix. The rates of eating disorders in autistic and ADHD women are strikingly high, and they're often missed because the presentation doesn't look like the stereotype — the restriction might look like "I just forget to eat" (which is real and also sometimes a cover), the binge might look like "I can't stop snacking," and the sensory aversions might look like "I'm a picky eater" when the reality is a dangerous narrowing of acceptable foods.

If any of this resonates — if food occupies a lot of your mental space, if you're hiding what you eat, if you feel out of control around food, if the thought of eating is scary, if weight and shape are a constant background hum — please treat this as medical information, not a chapter to finish: eating disorders are real, they're common in our community, and they're treatable. The systems in this chapter are not a substitute for professional help, and no shame attaches to needing it. Asking for help with food is not a failure of the food system. It's the food system recognising its limits, which is exactly what good systems do.

For everyone else — and for you, on the days when food is just food — the takeaway is this: you are not broken because you forget to eat. Your brain was not designed to notice hunger, plan meals, or resist dopamine. The system

was. Build the autopilot, route the snacks, honour the sensory list, and let go of the perfect diet that never survived contact with a real week. Eating — adequately, acceptably, without shame — is not a moral achievement. It's infrastructure. And infrastructure is meant to be built, not earned.

Toolkit Summary

- The AuDHD food cycle is mechanical, not moral: forgetting to eat, hyperfocus that blocks meals, sensory food aversions, and dopamine-seeking snacks that crash your blood sugar.
- The costs are physical and emotional: energy crashes, blood-sugar-driven mood swings that look like mood disorders, and shame about the way you actually eat.
- The goal is not a perfect diet — it's eating consistently without requiring executive function. Build autopilot food: a short list of sensory-acceptable, always-available, zero-preparation foods, eaten on repeat.
- Make snacks count: pair the dopamine food with a protein food so the crash doesn't land, and use the dopamine-free protein trick (a shake, a glass of milk, ham from the packet) when eating feels impossible.

- Treat your sensory-friendly food list as a legitimate document, optimise inside it, and let the frozen aisle be a system, not a fallback.
- Redesign meal prep around moments of capability, not Sunday schedules: prep when you can, in a limited set of moves, and keep a drawer of always-fine foods for the days cooking isn't happening.
- Eating disorders are common in AuDHD women and often missed because they don't look like the stereotype. If food is taking over your mental space, seek professional help — it's treatable, and no shame attaches to asking.

Chapter 5: Movement That Doesn't Require Motivation

SOMEWHERE IN YOUR HOUSE, POSSIBLY IN A DRAWER, POSSIBLY IN A CUPBOARD, POSSIBLY IN A GYM BAG THAT HAS BECOME A STORAGE CONTAINER, THERE IS A PIECE OF EXERCISE EQUIPMENT. IT MIGHT BE A YOGA MAT. IT MIGHT BE A PAIR OF RUNNING SHOES. IT MIGHT BE A MEMBERSHIP CARD FOR A GYM YOU JOINED WITH GENUINE ENTHUSIASM IN JANUARY AND HAVE VISITED — LET'S BE HONEST WITH OURSELVES HERE — APPROXIMATELY FOUR TIMES. YOU ARE NOT BAD AT EXERCISE. YOU ARE NOT LAZY. YOU ARE A PERSON WHO HAS BEEN SOLD A MODEL OF EXERCISE THAT IS STRUCTURALLY INCOMPATIBLE WITH YOUR BRAIN, AND THEN BLAMED YOURSELF FOR THE INCOMPATIBILITY. THIS CHAPTER IS ABOUT THROWING THAT MODEL AWAY AND BUILDING A DIFFERENT ONE.

Here's the model you were sold: exercise is a scheduled, planned, equipment-requiring, hour-long, sweat-producing, gym-based activity that you do three to five times a week, and it requires motivation. Here's the truth about that model: it requires planning (choosing the time, the class, the route), gear (the right shoes, the right clothes, the bag), travel (getting to the gym or the park), a sustained block of time (an hour, which for your brain is an eternity), and a sustained internal state called

motivation, which for an AuDHD brain is a weather system — it shows up unpredictably, stays briefly, and cannot be summoned on command. The model fails not because you fail, but because it was designed for a brain that plans ahead, tolerates boredom, and can make itself do things it doesn't want to do. Your brain is not that brain. So we're not going to fix your brain. We're going to fix the model.

Why "Exercise" Fails

Let's name the specific failure points, because they're all addressable once you see them. **Planning:** exercise-as-scheduled-activity requires you to decide in advance and then execute at the appointed time — two executive function tasks, both of which your brain is bad at, one of which (executing at the appointed time) requires transitioning between contexts, which your brain taxes heavily. **Gear:** exercise has a gear threshold. The shoes, the clothes, the sports bra, the bag — every item is a step, and the threshold is cumulative. If the sports bra is in the wash, the run doesn't happen. That's not an excuse; that's how thresholds work, and yours is lower than most. **Sustained motivation:** an hour of exercise is an hour of low-stimulation repetition, and your brain runs on novelty. The third minute of jogging and the thirtieth minute of jogging feel identical, and your brain experiences that as a deprivation — so it starts negotiating: "this is boring, we could stop, what if we stopped, is anyone watching us

stop, we could get a coffee instead." The negotiation is not weakness. It's your dopamine system doing its job, which is to seek novelty, in an environment that offers none.

And underneath all of it sits the deepest problem: exercise, as sold, is a means to an end. You do it now for a payoff later — fitness, weight, health, longevity. Your brain does not process delayed payoffs; it processes immediate ones. The reward for the run is a vague future benefit, and the cost is an hour of present discomfort. For a brain that runs on immediacy, that trade is a losing proposition every time, which is why the membership card gathers dust. You haven't failed to exercise. You've failed to find a version of movement that pays out now.

Walking: The Underrated AuDHD Exercise

The good news is that such a version exists, and it's the most underrated activity in the entire health canon: walking. Walking is the AuDHD exercise not because it's easy, but because it has exactly the properties your brain needs. **It requires no planning:** you can do it from your front door, in the clothes you're wearing, with zero gear, at any moment the impulse strikes. **It has no threshold:** there's no sports bra problem, no bag, no booking — the gap between thinking "I could walk" and walking is about ninety seconds. **It provides input:** walking is movement through a changing sensory world — sights, sounds, weather, people, dogs — which feeds your under-

stimulated brain while your body moves. It is, in fact, one of the few activities where your autistic half and your ADHD half agree: the ADHD half gets the stimulation, the autistic half gets the predictable rhythm of the steps. **It pays out now:** the immediate effects — the shift in state, the sensory input, the feeling of having done something — arrive during the walk, not months later. And it has a secret superpower for AuDHD brains: it's a thinking activity. Some of our best processing happens while walking, because the rhythm of the steps settles the body enough that the mind can actually work. The walk that "counts" as exercise is also the walk that untangles the problem, processes the conversation, and returns you home a different person than the one who left.

The only requirement is to remove the barriers that turn walking into exercise-as-sold. Don't schedule it — catch it. When you feel restless, when you're on the phone, when you need a break from the screen, walk. Ten minutes counts. Five counts. The "I only walked for fifteen minutes, that's pathetic" voice is the exercise-as-sold voice, and it's not allowed in this system. Walking for fifteen minutes, most days, without gear, without planning, without shame, is a functioning movement system. It's the foundation.

Movement Snacks

The second pillar: movement snacks. The principle is the same one that runs through the whole book — smaller units, lower thresholds, no executive function required. A movement snack is any one-to-five-minute burst of movement, done in the moment, with no gear and no transition: ten squats while the kettle boils, a minute of stretching when you stand up from the desk, walking up and down the stairs twice, dancing to one song, a plank during a loading screen, lunges across the kitchen. The snacks add up — a dozen two-minute snacks across a day is real movement — and more importantly, they keep your body in the habit of moving, which is the habit that actually matters.

Why snacks work where sessions fail: they have no planning requirement (you don't decide in advance, you catch the moment), no sustained-motivation requirement (any brain can tolerate two minutes), and they're triggered by the environment rather than by willpower (the kettle, the standing-up, the ad break — these are the alarms). You are not replacing exercise with snacks; you're replacing the exercise-as-sold model with a movement system that your brain can actually run. If a full session happens — great, it's a bonus, not the requirement. The system runs on snacks and walks. Everything else is gravy.

Body Doubling and Stimming-Adjacent Movement

Two more tools from the AuDHD toolkit, adapted for movement. **Body doubling:** the same presence-based magic that works for admin works for movement. If you can't make yourself walk alone, walk with someone — a friend, a partner, a dog, a podcast that feels like company, a body-doubling app where a stranger walks alongside you on screen. The presence outsources the initiation: you're not deciding to exercise, you're meeting a person, and the walk is what happens while you're there. This is not cheating; it's architecture. The social brain runs the movement brain, which is exactly what the social brain is for.

Stimming-adjacent movement: this one is about honouring what your body already does. Stimming — the repetitive movements your body uses to regulate — is movement, and it's a signpost: it tells you what kinds of movement your nervous system actually craves. If you rock, you might love a rocking chair or a swing. If you pace, walking is already your native movement — you've been an exerciser your whole life, you were just doing it without the membership. If you bounce your leg, you might love a rebounder or a dance class. The insight is to stop importing movement your body doesn't want and start exporting the movement your body already does. Pacing is walking. Rocking is core work. The stim is not a habit to be suppressed; it's a movement preference to be

honoured, and honouring it removes the entire motivational problem, because the movement is already happening.

Novelty: The ADHD Fuel

Your brain runs on novelty, so feed it novelty. The exercise-as-sold model fails partly because it's repetitive — the same run, the same class, the same machines, forever — and repetition is the one thing your brain cannot sustain. The fix is to make novelty a feature of the system, not a failure of it. You don't have to find your one perfect activity and stick to it for life. You have to have a rotating menu. Dance one week, climbing the next, a long walk the third, a YouTube workout you've never tried the fourth, swimming when you can get to a pool, the trampoline, the hula hoop, the video game that makes you move, the sport you played once in school and remember enjoying. The rotation is the system — the novelty is what keeps you coming back, and the coming back is what matters.

There's also a specific version of novelty that works especially well: exercise that doesn't look like exercise. The dance class that's actually a night out. The bike ride that's actually transport. The gardening that's actually an upper-body workout. The Ikea trip that's actually ten thousand steps. Movement that's disguised as life doesn't trigger the "I should exercise" resistance, because it's not exercise — it's just what you're doing. Count it. It counts.

Sensory Considerations

And now the autistic half's vote, because it's often the silent reason the gym membership failed. Gyms are sensory environments designed by people who do not share your sensory system: the lights are bright and fluorescent, the music is loud and wrong, there are mirrors everywhere, there's the smell of other people's sweat layered over cleaning chemicals, there are machines clanking and people grunting and televisions you can't follow, and the changing room requires a level of social performance that could fuel a small theatre. None of this is your imagination. It's a real sensory load, and it's a legitimate reason to never set foot in a gym again. You are not required to exercise in a gym. The gym is not the only temple of movement; it's just the one with the marketing budget.

If you do want to move indoors, choose the least hostile option: a pool (water dampens sound and smell and provides deep pressure — for many of us the most sensory-friendly exercise environment that exists), an empty park in the morning, a room in your own house with the curtains closed and your own music. And sort the fabric problem, because it's real: the wrong fabric — the scratchy seam, the synthetic that doesn't breathe, the tag — will end the workout before the fatigue does. You are allowed to own exactly one set of exercise clothes that feel acceptable, and to wear them until they disintegrate. Buy a second identical set so the wash cycle never

cancels a walk. Fabric is not vanity; fabric is a sensory gate, and the gate has to open before the movement can happen.

Movement as a System, Not a Goal

Here's the final reframe, and it's the one that makes all the others stick: the goal is not a fitness goal. The goal is a movement system — a set of defaults that keeps your body moving without requiring you to be motivated, organised, or good. The system has no finish line, which is exactly why it can't be failed. You can't fail a system that runs on walks and snacks and novel attempts and the occasional accidental ten thousand steps. You can only run it, imperfectly, most days, which is the whole point.

And rest days — take them without guilt, and without the voice that says rest is a failure. Rest is not the absence of the system; rest is part of the system. Your body needs recovery, your nervous system needs the quiet, and your relationship with movement needs to not be a punishment ledger where every missed day is a mark against you. The ledger is the old model. The new model is: move when you can, in ways that feel like life, and rest without apology. That's not a soft option. It's a sustainable one — and sustainability is the only thing that's ever going to beat the gym membership you bought in January.

Toolkit Summary

- The exercise-as-sold model (planned, geared, hour-long, motivation-required) is structurally incompatible with the AuDHD brain — planning, thresholds, boredom, and delayed payoffs all work against you. The model fails, not you.
- Walking is the underrated AuDHD exercise: no planning, no gear threshold, sensory input while moving, immediate payoff, and it doubles as a thinking activity. Catch it rather than scheduling it, and don't let the "only fifteen minutes" voice disqualify it.
- Movement snacks — one-to-five-minute bursts triggered by the environment (kettle, standing up, ad breaks) — build the movement habit without executive function.
- Use body doubling for movement (walk with a person, a dog, or a virtual companion) and honour stimulating-adjacent movement: pacing is walking, rocking is core work. Export the movement your body already does.
- Feed novelty: a rotating menu of activities, plus movement disguised as life (dancing, cycling as transport, gardening). Novelty is what keeps you coming back.

- Respect the sensory vote: skip the gym if it overwhelms, consider the pool, and solve the fabric problem — one acceptable outfit, bought twice.
- The goal is a movement system, not a fitness goal: walks, snacks, novelty, and rest days taken without guilt. You can't fail a system with no finish line.

Chapter 6: Hormones, Cycles, and the AuDHD Body

HERE'S A QUESTION YOU MAY NEVER HAVE BEEN ASKED, BY ANY DOCTOR, EVER: DO YOUR ADHD SYMPTOMS CHANGE WITH YOUR CYCLE? BECAUSE FOR MANY OF US, THE ANSWER IS PROFOUNDLY YES — AND THE FACT THAT NOBODY ASKS IS ONE OF THE GREAT GAPS IN WOMEN'S HEALTH. THINK BACK OVER RECENT MONTHS AND SEE IF YOU RECOGNISE THIS SHAPE. THERE'S A WEEK — USUALLY THE WEEK BEFORE YOUR PERIOD — WHEN YOUR MEDICATION SEEMS TO STOP WORKING. THE SAME DOSE THAT CARRIED YOU THROUGH THE REST OF THE MONTH SUDDENLY DOES NOTHING. YOUR EXECUTIVE FUNCTION COLLAPSES, YOUR EMOTIONAL REGULATION FLOODS AT THE SLIGHTEST PROVOCATION, YOUR SENSORY TOLERANCE EVAPORATES — EVERYTHING IS TOO LOUD, TOO BRIGHT, TOO MUCH. YOU'RE NOT IMAGINING IT. YOU'RE NOT DOING ANYTHING WRONG. YOU ARE LIVING IN A BODY WHERE THE HORMONES AND THE NEUROTYPED ARE TALKING TO EACH OTHER — AND NOBODY TOLD YOU THE CONVERSATION WAS HAPPENING.

This chapter is about that conversation — the layer of AuDHD health that gets ignored: by a medical system that treats cycles as a gynaecological issue rather than a neurological one, and by a community that focuses on the brain while the body runs its own monthly programme.

But it modulates everything else in this book. Sleep, food, movement, executive function — all of it shifts with your cycle, and understanding the shifts is the difference between fighting your body and working with it.

The Cycle and the ADHD Brain

Let's start with the mechanism, because it's elegant and it explains so much. Oestrogen is not just a reproductive hormone — it's a brain hormone. It affects dopamine and serotonin systems directly: oestrogen supports the dopamine signalling that your ADHD brain is already running low on. This means that when your oestrogen is high — roughly the week or two after your period — your ADHD symptoms are at their mildest: the medication works better, the executive function is a little more available, the world is a little more manageable. And when oestrogen falls — the week or two before your period — it takes the dopamine support with it, and your symptoms get louder exactly when the hormone that helps them is quiet. The week before your period isn't "PMS on top of ADHD." It's ADHD on top of ADHD, because the oestrogen dip temporarily removes the little natural support your brain had.

This is why the same dose can feel like a miracle in week two and a placebo in week four — the baseline it's working on has moved. For some women, a slightly higher dose for the premenstrual week, or a luteal-phase "boost" prescribed by a clinician who understands the interaction,

helps considerably. If your prescriber has never heard of this, you're not wrong to ask — you're better informed than they are.

PMDD: The Overlap Nobody Talks About

Now the serious one. Premenstrual dysphoric disorder — PMDD — is a severe, biologically distinct form of premenstrual syndrome that affects a small percentage of women in the general population, and a strikingly high percentage of women with ADHD and autism. The research is young, but the clinical picture is consistent: AuDHD women are overrepresented among PMDD sufferers, and PMDD is underdiagnosed in AuDHD women, because the symptoms — severe mood swings, irritability, depression, anxiety, rage, sensory intolerance, cognitive fog — look so much like "just" AuDHD and "just" PMS that nobody separates them out.

PMDD is not bad PMS. PMS is uncomfortable; PMDD is disabling. It's a cyclical, hormone-triggered mood disorder where for one to two weeks of every month the emotional system goes haywire: thoughts of self-harm, crushing depression, rage that arrives fully formed. The suffering is real, it's biochemical, and it's treatable. If you recognise this in yourself, hear this clearly: it is not a personality flaw and not something to endure quietly. It's a medical condition, and the first step is tracking — you

need to show the pattern is cyclical, because cyclicity is the diagnostic signature, and it's exactly what your time-blind brain won't notice on its own.

The Cycle You Can Actually See

Which brings us to the first and most important strategy: tracking. You cannot manage what you cannot see — and to your brain, the luteal phase is not a calendar event but a mysterious fortnight of misery that passes and is forgotten until it returns. Tracking converts the mystery into data, and data into a pattern, and a pattern into a plan.

The tracking doesn't need to be elaborate — a period app, a paper calendar, a notes entry; use whatever sticks. What you're looking for is the shape: which days of your cycle are the hard ones, which are the easy ones. Track for two or three cycles, and track the basics: period days, plus a daily one-to-ten rating of mood, energy, sensory tolerance and how well your medication feels like it's working. The point is not data science. The point is evidence that lets you predict the bad week instead of being ambushed by it — and show a clinician something concrete instead of performing your symptoms from memory.

The Hormone-Aware Schedule

Once you can see the cycle, you can schedule around it — and this is where the tracking pays off. The hormone-aware schedule is simple: put the hard things in the good weeks, and put the soft things in the hard week. The week after your period — oestrogen rising, the world more manageable — is for the things that need executive function: appointments, admin, big tasks, difficult conversations, new routines. The week before your period — everything harder — is for autopilot: easy food, gentle movement, extra sleep, social plans cancelled without guilt, the to-do list reduced to essentials. Lowering the bar in the hard week isn't weakness; it's accuracy about your capacity, and accuracy is what keeps you from pushing through a week your body has already written off.

The hormone-aware schedule extends to the other systems in this book. The luteal phase is not the time to start a new sleep routine, food system or movement habit — start those in the good week, so they're established enough to carry you through the bad one. And be gentle with missed days: the walk you skipped in the hard week isn't a failure of your movement system; it's the system being hormone-aware. The schedule bends, and the bend is the design.

Hormonal Contraception: Choices and Trade-offs

The contraception question is where the hormonal layer gets personal: there's no universal right answer, only the right answer for your brain and body, and the trade-offs are real. The combined pill, the patch and the vaginal ring all deliver steady hormones and can be taken continuously (skipping the placebo week) to reduce or eliminate periods — and for many AuDHD women, the single biggest hormonal win is simply not having a cycle at all. No cyclical mood crash, no weekly executive collapse, no PMDD fortnight. If your cycle is destroying your quality of life, continuous combined contraception is a legitimate, common, doctor-supported intervention — not a fringe choice.

The progestogen-only methods — the mini-pill, the implant, the injection, the hormonal coil — are a more complicated story for our community, because progestogen affects mood and cognition badly in some people, and the effects are highly individual. For one woman the coil is life-changingly neutral; for another the mini-pill is months of depression. The research can't tell you which you'll be, because research is averages and you're an individual. What it can tell you: if a hormonal method makes your mood, sensory tolerance or ADHD symptoms worse, that's a real side effect — not something to push through for six months. Three months is a reasonable trial; after that, if it's worse, it's wrong for

you, and you're allowed to switch. Non-hormonal options exist too — the copper coil, barrier methods, fertility-awareness-based methods (which pair beautifully with the tracking you're already doing) — and for women who are sensitive to synthetic hormones, they're not a compromise, they're a solution.

Any conversation about contraception and your brain belongs in the same room. When you discuss options, ask the direct question: "How will this affect my ADHD symptoms and my mood?" A clinician who dismisses it is a clinician to move on from. Your brain is part of the equation. It always has been.

Perimenopause: The Rewiring

And then there's the big one, the change that the whole medical system under-serves and our community barely discusses until it arrives: perimenopause. Perimenopause — the years-long transition before menopause, usually starting in the forties, sometimes earlier — is a profound neurological event for every woman, and a seismic one for an AuDHD brain. Oestrogen, the hormone quietly supporting your dopamine system for decades, begins to fluctuate and then fall, and ADHD symptoms that were managed for years suddenly come roaring back: executive function collapses, memory goes, emotional regulation destabilises, sensory tolerance drops, sleep fragments, anxiety spikes. Some women get their first ADHD diagnosis in perimenopause, because the symptoms

finally become too loud to ignore; others find their medication suddenly inadequate. It is a rewiring — the brain you had been managing becomes a different brain, mid-stride, without asking your permission.

The essentials belong here, because perimenopause is not a distant hypothetical — it's the reason many women reading this are struggling without knowing why. Perimenopause is a medical event, not a moral one, and it is treatable. Modern hormone replacement therapy — especially body-identical oestrogen via patch or gel — is safe and effective for most women, and often dramatically restorative, including for the brain: many AuDHD women find HRT restores cognitive function they'd forgotten they had, and some find their ADHD medication works again once the oestrogen floor is back under it. If you're in your forties (or late thirties) and your symptoms have suddenly worsened — the brain fog, the mood, the memory, the sleep — perimenopause belongs on your list of suspects. Don't accept "just stressed." The hormonal layer is real, it's neurological, and it's yours to advocate for.

Autism and the Hormonal Shifts

The hormonal layer has an autistic face too. If you live on routines, hormonal shifts are routine disruption from the inside. The sensory changes are real: sensory tolerance drops in the luteal phase, smells become intolerable, touch — even from loved ones — becomes overwhelming, the texture of food becomes a barrier. The

body itself becomes unpredictable — bloating, pain, temperature swings — and unpredictability is what your autistic nervous system treats as a threat. Add the social layer: the premenstrual week makes the mask harder to hold, and the mask failing is itself a source of distress, because you can feel the damage happening and cannot stop it.

The strategies are the same ones that run through this book, applied to the hormonal week. **Protect the routine:** in the hard week, keep the schedule simpler and more rigid — sameness is soothing, and the hormonal disruption already provides enough novelty. **Protect the senses:** pre-empt the intolerance — the earplugs, the safe clothes, the quiet evenings, the safe-list food. **Protect the alone time:** the luteal phase is not the week to be social; the mask needs more energy, so budget for unmasked time like you'd budget for sleep. And **lower the expectations with love:** the autistic half of you wants the plan to hold, and the hormonal week breaks plans; the answer is a pre-negotiated, hormone-aware plan designed to bend — one that includes permission to cancel, to rest, to be less, because the plan itself knows what week it is.

Medication and the Cycle

Finally, the interaction that ties it together: your medication and your cycle. Two facts to hold: (1) stimulant medication can affect your cycle — some women find their periods change or their PMS worsens on

stimulants, and if that happens it's worth discussing, because the dose or timing can often be adjusted; and (2) your cycle affects your medication — the luteal-phase oestrogen dip can make the same dose feel weaker, which is the phenomenon we started this chapter with. If you track both — the cycle and the medication effectiveness — you'll have the data for the connected conversations: with your prescriber about a luteal-phase boost, with your gynaecologist about hormonal options, with your GP about HRT if you're in perimenopause. The three conversations are connected, and you are the only person who will connect them.

The hormonal layer of AuDHD health is the layer that was never explained to you, and that's not your fault — it's a gap in the system. But the gap is closable, and you're the one who can close it: track the cycle, schedule around it, treat the hard week with the same compassion you'd give a friend, and take the hormonal questions — contraception, PMDD, perimenopause — to the clinicians with evidence in hand. Your hormones are not a separate problem from your AuDHD. They're the volume knob on it. And volume knobs, once you find them, can be turned.

Toolkit Summary

- Oestrogen supports the dopamine systems your ADHD brain runs low on: when it's high (post-period), symptoms are milder; when it falls

(premenstrual), ADHD gets louder and medication can feel weaker. That's a real interaction, not a failure.

- PMDD is a disabling, cyclical mood disorder that's overrepresented in AuDHD women and underdiagnosed because it looks like "just" AuDHD and "just" PMS. It's biochemical and treatable — track it and take it to a clinician.
- Track your cycle for two to three months: period days plus daily ratings of mood, energy, sensory tolerance and medication effectiveness. The data converts mysterious misery into a predictable pattern.
- Build a hormone-aware schedule: hard tasks in the good weeks, autopilot in the hard week — and never start a new routine in the luteal phase. Lowering the bar in the hard week is accuracy, not weakness.
- For contraception, there's no universal answer: continuous combined methods can eliminate the cycle entirely (a legitimate win for many); progestogen-only methods are highly individual — trial for three months, and if mood or cognition worsen, switch. Non-hormonal options are solutions, not compromises.
- Perimenopause is a neurological rewiring that hits AuDHD brains hard: symptoms return, memory and mood destabilise, and new

diagnoses are common. It's treatable — HRT is safe and often restorative, including for cognition. Don't accept "just stress."

- The autistic layer: hormonal shifts disrupt routines and senses from the inside — protect the routine, the senses and the alone time in the hard week, and build a plan designed to bend.
- Track medication and cycle together so you can have the connected conversations: a luteal-phase dose boost, hormonal options, and HRT if relevant. Your hormones are the volume knob on your AuDHD — find it and turn it.

Chapter 7: Medical Appointments Without the Dread

YOU HAVE BEEN MEANING TO BOOK THIS APPOINTMENT FOR SIX WEEKS. YOU'VE REHEARSED WHAT YOU'LL SAY — AND LOST BOTH NOTES. YET EVERY TIME YOU OPEN YOUR PHONE TO CALL, SOMETHING HAPPENS — YOU GLANCE AT THE TIME, YOU REMEMBER SOMETHING ELSE, YOUR CHEST TIGHTENS, AND YOU PUT THE PHONE DOWN "FOR A MINUTE." THE MINUTE BECOMES A WEEK, THE WEEK SIX. THE APPOINTMENT IS NO LONGER JUST UNBOOKED; IT'S HAUNTED — A TASK THAT HAS GROWN HEAVIER WITH EVERY DAY YOU HAVEN'T DONE IT. WHEN YOU FINALLY BOOK IT, YOU'LL SPEND THE FORTNIGHT BEFORE IN LOW-GRADE DREAD, AND THE APPOINTMENT ITSELF — FIFTEEN FLUORESCENT MINUTES WITH A STRANGER WHO HOLDS YOUR HEALTH IN THEIR HANDS — WILL BE ONE OF THE MOST EXHAUSTING PERFORMANCES OF YOUR MONTH.

This chapter is about dismantling that dread. Not by becoming braver — you're already brave enough to have survived every appointment you've ever had — but by changing the conditions. The dread is not a personality flaw. It's a predictable response to a situation that is genuinely, structurally hostile to your brain. When you

stop asking yourself to be a different kind of person and start changing what happens before, during and after the appointment, the dread loses its grip.

Why Appointments Are Uniquely Hard

Everyone finds medical appointments mildly stressful. That's not what we're talking about. We're talking about the specific, layered way they're hard for an AuDHD woman, and it's worth naming each layer, because each has a different fix and none of them is "just go."

The sensory gauntlet. For an autistic sensory system, a medical environment is an assault course designed by someone who has never met you. The waiting room: humming fluorescent lights, a television you can't follow, a crying child, chair fabric you can feel through your clothes, antiseptic layered over someone's perfume. Then the examination room: the paper gown, the too-bright ceiling light. By the time the doctor walks in, your nervous system has already spent its budget on surviving the room, and you have nothing left for the conversation. This isn't weakness; it's sensory load, and it's real. The fix isn't to endure it better — it's to reduce the load.

The communication gauntlet. The consultation is a test of skills your brain is worst at, administered in real time. The doctor asks open questions — "So, what brings you in today?" — and you must retrieve your internal experience from a body you have poor interoception for and organise it into a linear narrative, under social

pressure, while the clock runs. Working memory collapses under exactly this load. You forget the symptom you came for. You say "it's fine, it's probably nothing" because that's the sentence that ends the pressure fastest. And in the car park afterwards, you remember everything you meant to say — perfectly formed, now useless. The consultation isn't hard because you're inarticulate. It's hard because it's an executive function task with a time limit and an audience.

RSD, aimed at doctors. Rejection-sensitive dysphoria — the AuDHD lightning response to perceived criticism or dismissal — makes doctors uniquely terrifying, because they hold a particular power: the power to decide whether you're telling the truth. The fear isn't just of being disbelieved; it's of being judged dramatic, anxious, exaggerating. One raised eyebrow, one "have you considered that this might be stress?" and the RSD floods in: you're a fraud, you're wasting their time, you've made this up. The feeling is so overwhelming that avoidance becomes rational — why walk into a room where your reality might be dismissed, in the most polite professional way possible? You're not afraid of doctors. You're afraid of being dismissed, and you've been dismissed enough times to know it's not paranoia.

The "don't want to be a bother" conditioning. And underneath it all is the oldest layer: the conditioning that taught you, from girlhood, that your discomfort is an inconvenience to others. Don't make a fuss. Don't take up space. Don't complain about something that's probably

nothing. The doctor is busy; other people are sicker; you're fine, really. This conditioning is not yours — it was installed — and it's the most dangerous health belief you carry: it tells you your body's signals aren't worth another human being's time. Every appointment you skip "out of consideration" is the conditioning winning. The doctor's time is not more valuable than your life. You're not a bother; you're the point.

The Dread Cycle

Put those layers together and you get the cycle that governs most AuDHD women's healthcare: **delay** → **avoid** → **urgent** → **stress**. It starts with delay — the appointment feels big, so you put it off "until I have more energy." But energy never arrives on its own, and every day of delay adds shame to the task, making it bigger and easier to avoid. Then something forces the issue — the pain worsens, the prescription runs out — and the task becomes urgent. And here's the cruel part: urgency works. It's the one thing that reliably breaks executive paralysis. You get it made, in a panic, with zero preparation, and you walk into the gauntlet cold — and the bad appointment reinforces the dread, tightening the cycle for next time.

Notice what the cycle is doing: it's converting every appointment into a crisis, and crises are expensive. The fix isn't to break the cycle by willpower at the moment of urgency — willpower has already lost by then. The fix is

to interrupt it earlier, at the delay stage, with systems that make the appointment small enough that it never reaches urgent.

The Preparation System

The single most effective thing you can do for any appointment happens before you leave the house: writing down what you need to say. This sounds obvious. It is not obvious. Most people — including most doctors — assume you'll produce your symptoms and questions spontaneously in the room, from memory, under pressure. That assumption is exactly what fails you. So you stop playing by that rule. You write it down.

The symptom list. Before the appointment, in your own words, at your own pace, write down: what's bothering you; when it started (approximate is fine — "around my birthday" is a valid answer); what it feels like, in your own language — "like a stabbing pain behind my left eye," "like my brain is full of static"; what makes it worse and what makes it better; and how it affects your life — "I can't sleep on that side," "I'm missing work." The last one matters most, because doctors are trained to care about function: "It's stopping me from living my life" is the sentence that gets listened to. Write it in whatever format your brain tolerates — bullets, a voice note, a text to yourself. Then bring it. You're allowed to read from it,

or hand it over and say "I wrote this down because I forget things when I'm nervous." Every decent doctor will be grateful; you've just made their job easier.

The question script. Equally important: write your questions before you go, in your own words, not medicalised. "I want to know if this is normal." "Do I need to worry about this?" "What are my options?" "What happens next?" Three questions is plenty. Number them. Bring the paper. When the doctor asks "anything else?", you say "yes, I have three questions," and you work through them. The script exists because in the room, with RSD whispering, the pressure to say "no, that's all, thank you" is enormous — and the script says it for you.

The Support System

You don't have to do the room alone, and "alone" doesn't have to mean "solo." There are two kinds of support, and you can use either or both.

A support person. If you have someone who can come with you — a partner, a friend, a family member — they can do something specific: hold the list. Not hold you (though that's allowed too), but hold the paper. They take notes. They ask the question you froze on. They remember what the doctor said, because you won't — your working memory was occupied full-time with surviving the room. Tell them before you go what you need: "I need you to write down what the doctor says, and

ask my third question if I don't." That's a job description, and people do much better with job descriptions than with vague requests to "come for support."

Notes as support. Handing the doctor your written symptom list and question script isn't just communication — it's emotional scaffolding. The paper is a shield: when you freeze, it doesn't; when RSD floods in, it's evidence, in your own handwriting, that you're not making this up. Writing it down beforehand also calms the dread — the thing you were carrying in your head, where it could spin, is now on paper, where it just sits. The dread feeds on formlessness; written down, the appointment becomes a known quantity, and known quantities are much less frightening.

Sensory Preparation

You can't change the waiting room, but you can change what you bring into it. Sensory preparation is the art of carrying your own environment with you. The kit, chosen by you: earplugs or noise-cancelling headphones for the hum and the crying child; sunglasses for the fluorescents ("light-sensitive" is all anyone needs to hear); clothes you can actually tolerate — soft fabrics, no tags, layers for the heat of the room; a bottle of water and something to eat for afterwards (appointments are energy events); a fidget or a smooth stone for your hands while you wait; and your phone with a comfort show downloaded. If you can, book the first appointment of the morning or the first after

lunch, when the waiting room is emptiest. Some practices will let you wait in your car and text you when the doctor is ready — ask.

The Written Summary Request

Here's a sentence to practise, because it changes everything about the aftermath: "I find it hard to remember what's said in appointments — could you write down the main points, or send me a summary?" Say it at the start — it frees your brain from trying to memorise: the moment you know the information will be captured externally, you can stop the exhausting internal recording that ate your processing power, and actually listen. The written summary is also the antidote to car-park amnesia: instead of reconstructing the appointment from fragments and anxiety, you have the doctor's own words, in writing, to consult as many times as you need. And it's a record — useful for a second opinion later, or if a new doctor needs your history. It's a reasonable request, and you're entitled to a record of your own care.

Email-First Communication

Here's a strategy that sidesteps the entire in-person gauntlet for a surprising amount of healthcare: communicate in writing wherever you can. Most practices now have patient portals, online booking and email contact. Refill requests, bookings, follow-up questions,

test results, even some consultations — a great deal of routine care can happen without a phone call or a waiting room. This matters enormously for an AuDHD brain: asynchronous written communication plays to your strengths — you compose at your own pace, without performing composure, and you have a written record of everything. Email-first also reduces the RSD load — you're never put on the spot or talked over. Write your message like a mini symptom list — what's happening, when it started, how it affects you, what you're asking for — and send it. When a practice offers a portal, use it.

The Second Appointment Strategy

And finally, the strategy that releases the most pressure of all: you don't have to decide everything in one visit. Go into the first appointment expecting only to gather information, and schedule a follow-up for decisions. You don't have to agree to a treatment in the room, or choose between options under fluorescent light with RSD breathing down your neck. You can say the most powerful sentence in patient communication: "Thank you — I'd like to think about this and discuss it at a follow-up." That sentence isn't rudeness; it's how you protect your decision-making from the conditions that destroy it. Take the information home. Read it. Talk to your support person. Sleep on it — your brain does its best processing offline. Then, at the second appointment, you come back with your written questions, rested, prepared, and

deciding from information rather than pressure. And the second appointment fixes another problem: you now know the doctor, the room, the process — and the unknown, which is where dread lives, has been replaced by the known.

The Dread, Dismantled

None of this requires you to be braver, more articulate or less sensitive — it requires you to be better prepared, and preparation is a system, not a personality change. Preparation shrinks the appointment to your size; support means you don't hold it alone; sensory preparation lowers the room's load; writing protects your memory and your peace; the second appointment protects your decisions. Together they interrupt the dread cycle at the delay stage, so the appointment never has to become a crisis. The first time you walk out of a doctor's office having said everything you meant to say — with your questions answered, in writing, in your pocket — you'll feel something you may not have felt in years: that a medical appointment is just a conversation with a professional, and you're allowed to have it on your terms. That feeling is the system working. Keep the system, and it keeps working.

Toolkit Summary

- Appointments are hard for structural reasons, not character ones: sensory overwhelm, real-time communication under pressure, RSD around doctors, and girlhood conditioning that your discomfort is a bother.
- The dread cycle — delay → avoid → urgent → stress — converts every appointment into a crisis. Interrupt it early with systems, not willpower.
- Write the symptom list before you go: what, when it started, what it feels like in your own words, what makes it worse/better, and how it affects your life. Bring it, and read from it.
- Write three questions in your own words, number them, and work through them when the doctor asks "anything else?"
- Bring a support person with a job description (take notes, ask question three), or let the written notes be your support.
- Build your sensory kit: earplugs, sunglasses, tolerable clothes, water, food, a fidget, downloaded comfort viewing — and book the quietest appointment slots.
- Ask at the start of the appointment for a written summary or portal note — it frees your memory and gives you a record.

- Use email, portals and online booking wherever possible: asynchronous written communication plays to your strengths and leaves a paper trail.
- You don't have to decide in the room: "I'd like to think about this and discuss it at a follow-up" is a complete sentence. Use the second appointment for decisions.

Chapter 8: Medication Management on Both Fronts

THE SCENE IS SO COMMON IT'S PRACTICALLY A GENRE: IT'S TUESDAY, AND YOU REALISE — WITH THE PARTICULAR COLD FEELING OF A REALISATION THAT ARRIVES TOO LATE — THAT YOU'VE RUN OUT OF YOUR MEDICATION. NOT "RUNNING LOW." OUT. THE PACKET IS EMPTY IN YOUR HAND. YOU MEANT TO REFILL IT LAST WEEK — YOU EVEN HAD THE PHARMACY'S APP OPEN ONCE — BUT THE TASK NEVER QUITE HAPPENED, AND NOW YOU FACE TWO OPTIONS: DAYS WITHOUT THE MEDICATION THAT MAKES YOUR BRAIN WORK, OR A PANICKED PHONE CALL BEGGING FOR AN EMERGENCY PRESCRIPTION DURING WORK HOURS. EITHER WAY, YOU'VE LOST. THIS IS THE MEDICATION PROBLEM IN ITS PUREST FORM: THE CONDITION THE MEDICATION TREATS IS THE CONDITION THAT MAKES MANAGING IT ALMOST IMPOSSIBLE. ADHD MEDICATION IS MANAGED BY AN ADHD BRAIN. OF COURSE IT FALLS APART. IT'S NOT A MORAL FAILURE — IT'S A DESIGN FLAW, AND DESIGN FLAWS NEED DESIGN FIXES.

This chapter is the design fix. Medication is where the core idea of this book gets its sharpest test: managing medication is an executive function task, so the solution is a system, never guilt. We'll cover both fronts — practical and neurological — and build systems that keep them from collapsing.

Why Medication Is an Executive Function Gauntlet

Let's name what "just take your meds" glosses over. Remember to take them — which requires noticing the time, your blind spot. Take them at roughly the same time daily, because stimulants are timing-sensitive. Notice you're running low — prospective memory, your rarest resource — before it's an emergency. Request a refill — a phone call, a portal message, a pharmacy visit, or all three. Collect it, sometimes within a narrow window. And the part nobody warns you about: do all of this in a system that may have shortages, changed formulations, or a doctor who must review the prescription before reissue. Six steps, any one of which can break the chain — and the chain breaks the same way every time: silently, on a Tuesday, at the moment of need.

The shame spiral that follows — "I can't even manage my own medication" — is the real disease. It's the voice that says the system failure is a you failure. It isn't. The steps aren't hard because you're incompetent; they land on the executive functions you have least of, and they're invisible until they matter. So we stop trying to be better at the steps and build a system where they happen without you.

The Pill System

The foundation is the pill system: a way of taking medication that doesn't depend on remembering, because remembering is the step that fails. The classic tool is the weekly organizer — the seven-day box — and it works for a specific reason: it converts "did I take my meds?" from a memory question into a visual question. You don't have to recall; you look. The empty slot means taken; the full slot means not. For an AuDHD brain, externalising the memory is everything. Take multiple meds at different times? Get the four-slot version and load it once a week: ten minutes, a podcast, its own little appointment. And if you're prone to forgetting whether you've taken a dose, choose an organizer with a day-of-the-week column, so you can see Tuesday's slots are empty and know you haven't taken them — which no pill bottle will ever tell you. Where double-dosing is dangerous, the organizer is the single source of truth: no stray pill bottles anywhere, ever.

Alarms are the second layer. Set a daily alarm for each dose, named something you can't ignore: "TAKE MEDS — morning" rather than "alarm." But be honest about the limitation: if you're mid-flow when it fires — hyperfocused, mid-sentence — you'll silence it and forget within seconds. Pair the alarm with the next layer.

Visible meds are the third layer, and the one most people miss. Keep the medication where you cannot avoid seeing it at the time you need it: the morning dose next to

the kettle or the coffee machine, the evening dose on your pillow or by your toothbrush. Out of sight is genuinely out of mind for our brains — invisibility is deletion. The bottle in the bathroom cabinet might as well be in another country. Put the meds in your actual daily path and stack the dose onto something you already do — coffee, brushing teeth, feeding the cat. Habit-stacking is the most reliable medication-reminder technology ever invented, and it costs nothing.

If you miss a dose, the rule is simple: skip it, don't double up, and mention it at the next review if it's happening often. Missing doses is information, not confession.

The Refill Calendar

Taking the meds is only half the battle; the other half is the refill, and the refill is where the chain most often breaks. The refill calendar makes the refill a scheduled event instead of an emergency. Choose a fixed day — say, the first Monday of every month — and put a recurring calendar entry on it called "Medication check." The check takes five minutes: look at each medication, count what's left, and refill anything with less than two weeks' supply. The genius of the fixed day is that it replaces the impossible task — "notice when I'm running low, at the right moment, forever" — with a visible, recurring, do-it-on-this-day task. If your prescription requires a doctor's review before reissue (many ADHD prescriptions do, by

law), the medication check is also the moment to message the surgery, because the review itself can take days — and days of lead time are the difference between a routine refill and an urgent one. If your pharmacy offers automatic refills or mail-order, set it up. And a backup rule: when you open a new packet, immediately set a reminder to reorder two weeks before it runs out. New packet, new reminder — the anchor is physical, so it can't be forgotten.

The Medication Appointment

The refill calendar handles the supply; the medication appointment handles the quality of what's being supplied. Once or twice a year, book a dedicated appointment with your prescriber whose only purpose is to talk about how the medication is actually working — not to renew it in a rushed five-minute slot. Bring your tracking notes, bring your questions, and treat it like any important appointment: written list, prepared, no guilt. Ask the questions that never fit in a renewal call: Is this still the right dose? Would a split dose or a different release profile work better? Are these side effects normal — and is there an alternative? How does this interact with my other medications, my cycle, my sleep? The appointment matters because our medication needs change — with time, with hormonal shifts, with life circumstances — and only the prescriber can adjust it. Without it, the medication drifts: a dose that was right at diagnosis,

never revisited, slowly becoming wrong. The yearly medication appointment is the maintenance check for your brain's most important tool. If your prescriber is hostile to questions or dismissive of your experience, that's a signal about the prescriber, not about you — the chapter on navigating the system has more on what to do about that.

Tracking Effectiveness and Side Effects

To have a useful medication appointment, you need data — and your memory of "how the medication has been" is a time-blind, RSD-contaminated reconstruction, not data. So we track. One line per day in a notes app, three items: did I take it (and when), how effective did it feel on a one-to-five scale, and any side effects. That's it. Thirty seconds a day. Over a month, that line of daily data shows patterns your memory never will: effectiveness dipping in the week before your period, the afternoon crash worsening after a dose change, a headache that started exactly when you switched brands. Patterns are what prescribers act on, and patterns are what you can't see from inside the fog. Use an app if you prefer — the resources section lists good trackers — but the principle matters more than the tool: tiny, daily, boring data beats occasional dramatic recall. And track side effects without judgment: a side effect is information about whether this medication works for you, not evidence that you're failing.

The question is never "am I bad at medication?" but "is this medication right for me, at this dose, right now?" — data answers that, not shame.

The ADHD Medication Specifics

The specifics are worth knowing, because understanding your medication protects you from two failure modes: blaming yourself for effects that are normal, and tolerating problems that are fixable. Stimulant medication — the methylphenidate and amphetamine families — works by boosting the dopamine and noradrenaline signalling your ADHD brain runs low on. It's fast-acting: you feel it within the hour, and it wears off by day's end — take it too late and you'll be awake at midnight, take it inconsistently and you'll chase a moving baseline. The crash — the afternoon or evening slump as the dose wears off — is normal and manageable: a split dose, a smaller booster, or simply scheduling the crash (nothing important in that hour) are all legitimate strategies — discuss them with your prescriber. Tolerance is the other great worry — the fear that the meds have "stopped working." The reality is usually one of three things: the dose needs adjusting (normal, expected), something in your life changed the baseline (sleep, stress, hormones — the luteal-phase dip being the classic one), or you've adapted to the effects and are comparing yourself to an unrealistic memory of the first weeks. All three are conversations for the medication appointment,

not reasons to quit or to self-adjust — stimulants are powerful, and the person who adjusts them should be the person who knows your full picture. Non-stimulant ADHD medications are more forgiving on timing, but they build and fade slowly, and their effects shift over months — another reason the regular medication appointment matters.

The Autism Overlap: Sensory Side Effects and Routine Resistance

The second front is the one the ADHD literature rarely mentions: the autistic half of you is also on this medication. First, sensory side effects. Stimulants commonly suppress appetite — and for an autistic eater with narrow food options, that can make eating feel nearly impossible, triggering a cascade: low blood sugar, worse focus, worse mood, worse everything. The fix is mechanical, not motivational: eat before the meds kick in (even if it's the same safe food every day), and set a "protein snack" alarm for midday, because hunger won't remind you while the meds are working. Dry mouth, another common effect, can make food textures feel wrong; keep water by your side. Second, routine resistance. The autistic brain loves routines — and the ADHD brain can't maintain them — which means medication schedules sit right at the intersection of your two conditions: you crave the consistency the meds require, and you can't generate it. This is exactly why the

pill system isn't a nice-to-have but the whole point: it's the routine made physical, so the routine doesn't depend on you. And when the routine does break — the missed dose, the late refill — the autistic half feels the disruption as real distress, not a minor annoyance. The disruption is real, and the system, not you, is what needs repairing.

The Shortage Survival Plan

And now the part that has become a reality for too many of us: the shortage. Stimulant shortages have hit hard in recent years, and they're a special kind of horror for an AuDHD brain: an unpredictable disruption to the medication you depend on, with admin required to resolve it, during withdrawal. The shortage survival plan is preparation in advance, because you cannot think clearly about a shortage while you're already in one. First, know your options before you need them: ask your prescriber, at a calm moment, what the alternatives are if your usual medication is unavailable — a different brand, a different release profile, a different medication, a temporary dose change. Write the answers down. Second, build a legal buffer: if you can refill a few days early, do it, so you always have a small cushion. Third, know the rules: some jurisdictions let pharmacists substitute brands or offer emergency supplies without a new prescription — ask yours what they can do, in advance. Fourth, if a shortage hits, act the same day you learn you can't fill — alternatives take time to arrange, and the early request

gets the last stock. And fifth, have a plan for the gap itself — reduced demands, extra sleep, easy food, compassionate self-talk. A withdrawal plan written in advance, by the version of you who could think clearly, is a gift to the version of you who can't. The shortage is not your fault, and it's not a sign you're failing at medication. It's an external disruption to a system you've built — and systems, unlike willpower, can be rebuilt.

Both Fronts, One System

None of this requires you to be better at remembering, more disciplined, or less AuDHD — just setup, one afternoon, and then the system carries the load your brain was never designed to carry alone. The Tuesday-morning panic, the empty packet, the emergency phone call: these are not inevitable facts of life with medication. They're failures of design, and design is fixable. Your medication is one of the most important tools you have. It deserves a system as good as it is.

Toolkit Summary

- Medication management is an executive function gauntlet: remember, time it, notice it's running low, request, collect, repeat — six steps, any one of which can silently break. Systems, never guilt.

- The pill system: a weekly organizer (visual, not memory-based), named alarms, and meds placed visibly in your daily path, stacked onto existing habits. The organizer is the single source of truth; a missed dose means skip, don't double up.
- The refill calendar: a fixed monthly "medication check" day — count what's left, refill anything under two weeks, start doctor-review requests early (reviews take days).
- Book a yearly medication appointment with your prescriber, dedicated to how the medication is working — dose, timing, side effects, alternatives — with your tracking notes in hand.
- Track daily, tiny and boring: did I take it, effectiveness one-to-five, side effects. Patterns you can show beat memories you can't trust.
- Know the stimulant specifics: timing and crashes are normal; "it stopped working" is usually dose, baseline or expectation — conversations for your prescriber, never reasons to self-adjust.
- The autism overlap: appetite suppression needs mechanical fixes (eat before meds, a protein alarm), and routine breaks feel like real distress — repair the system, not yourself.
- The shortage survival plan, written before the shortage: alternatives from your prescriber, a legal buffer, pharmacy rules, act-same-day protocol, and a withdrawal plan for the gap.

- The whole thing is one system: tracking feeds the review, the review keeps the prescription current, the calendar keeps the supply flowing, the organizer keeps the dose in you. Set it up once.

Chapter 9: The Health System As an AuDHD Woman

HERE'S AN APPOINTMENT YOU MAY RECOGNISE: YOU WAITED SIX WEEKS FOR IT, PREPARED YOUR LIST, SURVIVED THE WAITING ROOM, AND EXPLAINED YOUR SYMPTOMS AS CLEARLY AS YOU COULD. AND THE DOCTOR LISTENED, NODDED, AND SAID THE SENTENCE: "IT'S PROBABLY JUST STRESS. HAVE YOU TRIED RELAXING?" OR: "YOUR BLOOD TESTS ARE NORMAL, SO YOU'RE FINE." YOU LEFT CARRYING THE STRANGE, FAMILIAR FEELING: THAT WHAT JUST HAPPENED DIDN'T REALLY HAPPEN — THAT THE PAIN ISN'T REAL, OR ISN'T WORTH INVESTIGATING, OR IS SOMEHOW YOUR FAULT FOR EXPLAINING IT BADLY. YOU QUESTIONED YOUR OWN BODY ON THE WAY HOME. YOU DECIDED THE PROBLEM WAS YOU.

This chapter is about that experience — being an AuDHD woman inside a healthcare system that was not built for you — and what to do about it. The gaslighting is a system problem, not a verdict on you, and its countermeasures — self-advocacy, documentation, second opinions, advocates — are skills, not personality traits. Skills can be learned.

The Gaslighting Problem

Let's name what's happening, because naming removes some of its power. Women's symptoms are dismissed at a rate that's now well documented: more likely to be told physical symptoms are anxiety or stress, more likely to be diagnosed with a mental health condition instead of a physical one, longer waits for correct diagnosis of heart disease, autoimmune conditions and more. It's not that doctors are monsters. Medicine was built on a default patient who is male — female bodies are the deviation, and deviations get labelled vague, emotional or atypical. Add the socialisation that teaches women to present symptoms apologetically — "I'm probably overreacting, but..." — and you have a machine for turning real illness into "it's probably nothing."

Now add the AuDHD layer, and the dismissal gets a second mechanism. Your symptoms don't arrive in the linear, one-problem-at-a-time format the system expects. They arrive as overwhelm: sensory overload, fatigue, brain fog, anxiety, physical complaints that shift and blur. To the doctor, this looks like anxiety — a box they can close quickly. "It's just anxiety" is the most dangerous sentence in women's healthcare, because it can be true and still be wrong: you may well have anxiety — AuDHD women usually do — and it may be a symptom of an uninvestigated physical problem, or a by-product of living unsupported, or both. The doctor closes the file, and the real problem — the thyroid, the deficiency, the burnout —

goes underground for another year. You're not paranoid. This is a documented pattern, applied to you twice: once for being a woman, once for being neurodivergent.

The Doubly Dismissed

And the second layer has a specific flavour. Autistic presentation may not match doctorly expectations: no eye contact, pain described factually without the distress performance clinicians are trained to look for, or masking so hard you seem fine. ADHD means you may forget details or get the timeline wrong — read as unreliability, not neurology. The combination is devastating: the very traits that make you AuDHD — the honest flatness, the scattered delivery, the sensitivity — read to a busy clinician as exaggeration or hysteria. And the RSD response to being disbelieved — "I'm making this up, I'm a burden" — does the system's work for it. You leave and don't come back, which is exactly what a system that doesn't want to investigate does.

So let's set the record straight, and hear this: when a doctor dismisses you, that is a data point about the doctor and the system, not about the truth of your body. Your symptoms are real. They happened. A normal test result means the test was normal, nothing more; it does not mean nothing is wrong. You are the only person who has continuous access to your body. The doctor has fifteen minutes. Your experience outranks their dismissal.

Self-Advocacy as a Skill, Not a Personality

Self-advocacy gets talked about as if it's a character trait — you either have it or you don't. That's a particularly cruel framing for AuDHD women, who were taught their whole lives that they're too much and should pipe down. Self-advocacy is a skill, like reading a map, with components that can be learned: **knowing what you want** (the preparation system — symptom list, question script); **saying it in one clear sentence** (below); **persisting** (asking twice, in writing, without apology — "I understand the test was normal; I'd still like to discuss what could explain these symptoms"); **documenting** (a record of every appointment, call and result, so your history isn't hostage to your memory); and **escalating** (knowing when to ask for the next person up). None of this requires charisma — it requires scripts and repetition. Practise at home, in writing, before you need them. By the time you're in the room, the sentence is already built.

How to Ask for What You Need Clearly

The heart of self-advocacy is the clear ask, and there's a formula that works across every consultation: **symptom + impact + request**. "I've had chest pain when I climb stairs for three months, it's stopping me walking to work, and I'd like it investigated." "I'm exhausted all the time, it's affecting my job, and I'd like my thyroid and iron checked." The formula leads with facts, connects them to

your life (what clinicians are trained to act on), and ends with a specific request — hard to deflect, where "I feel terrible, please help me" is easy to deflect. Two more sentences to have ready. First: "I'd like that noted in my records," said whenever a concern is dismissed. It's your right, and it changes the dynamic: a doctor who knows their response will be on record thinks harder about it. Second, the empathy question, used sparingly and only after dismissal: "What would you recommend if I were your sister?" It forces the clinician to check whether their answer changes when the patient matters to them personally.

The Second Opinion Permission

Here's a permission slip you may never have been given: you are allowed a second opinion. Not just for serious diagnoses — for any situation where you feel unheard, uncertain, or the stakes are high. It's not an insult to the first doctor; it's standard practice, your right in most systems, and the best clinicians welcome it — the ones who resent it usually have something to resent. Ask your current doctor to refer you ("I'd like a second opinion on this"), or self-refer where your system allows. Bring your records — results, the written summary from the last appointment, your symptom list — so the new doctor isn't starting from scratch. And the reframe that makes this usable: a second opinion is not a verdict on your credibility. It's more information, gathered like you'd

gather it for any big decision. Both doctors agree? You have certainty, which is worth something. They disagree? You have a conversation — better than silence either way.

Understanding Your Rights

Rights are the ground the whole system stands on, and knowing yours changes how you walk through it. The precise details vary by country, but the universal ones are these: **the right to your records** — you can request copies of your records, results and referral letters, no justification needed; **the right to informed consent** — you can't be treated without understanding what and why, and you can say no; **the right to a second opinion; the right to choose and change providers**, without needing permission; and **the right to be accompanied** — bring someone to any appointment, and request an advocate or translator where available. The specifics are searchable for your country — your health service's patient rights page is worth an afternoon. Hold the principle: the system serves you, not the other way around. You are not a supplicant asking for favours; you're a customer, and customers get to ask questions, get a second estimate and take their business elsewhere.

Finding Neurodivergent-Affirming Providers

The best advocacy move is choosing better providers in the first place — and the best question to ask any prospective one is: "Have you worked with autistic and ADHD adults? What's your approach to neurodivergence?" A neurodivergent-affirming provider doesn't just tolerate your AuDHD; they see it as relevant to your care — they ask about sensory needs, are comfortable with you bringing notes, don't read flat affect as depression or scattered speech as exaggeration, and take your side-effect reports seriously. Directories exist (the resources section lists them), and asking around in AuDHD communities — "anyone know a dentist who's good with sensory stuff?" — is how most of us find our people. It's worth travelling further for a provider who believes you. One good doctor is worth ten convenient ones.

The Referral Maze

The referral is where healthcare systems eat AuDHD women alive: it's a chain of tasks — identify the specialist, get referred, chase the referral, attend, chase results, get the next referral — and every link can silently vanish without anyone reminding you. The system for the maze is boring and essential: a referral tracker. One page containing who referred you, to whom, when, why, and

the expected wait. Set a calendar reminder for when it should have arrived, and if it hasn't, chase it with one line: "Following up on a referral from [date] to [specialist] — could you confirm it was received?" Chasing isn't nagging; it's the only way the maze works, because the system won't chase itself. And when a referral is refused or a result goes missing, the same rule applies: "Could you note that in my records, please?" — followed, if necessary, by the complaints pathway, which exists precisely for this. Complaining about care isn't drama; it's the system's feedback mechanism.

Insurance and Admin: The Paperwork System

In systems with insurance, claims and prior authorisations, the admin is a second full-time job nobody scheduled — and for an AuDHD brain, paperwork is where executive function goes to die. The countermeasure is the same as everywhere in this book: one place, one system, no memory required. A single yearly folder (physical or digital) for every document: insurance cards, claims, letters, referral confirmations, receipts, appeal deadlines. A standing monthly calendar entry — "Health admin" — to file what's accumulated and note deadlines. And the golden rule: if a form can be done online or by email, do it there; if it needs a phone call, schedule it in your calendar with the number and what

you need, so you're not doing call-roulette at 4pm on a Tuesday. Admin is not a test of your worth. It's a system, and systems are built, not born.

When to Bring an Advocate

And sometimes — more often than you think you're entitled to — you should not do any of this alone. An advocate is someone whose job is to be on your side: a friend with a job description (the support-person system from earlier), or a professional patient advocate where available. Bring one when the stakes are high (a new diagnosis, a treatment decision, a hospital stay); when you've been dismissed before; when you're exhausted, because exhaustion is exactly when the system eats you; or when you simply want one — that's a sufficient reason. The advocate's roles are specific: take notes, ask the questions you wrote but couldn't say, remember what the doctor said, and — critically — say the sentence you can't: "She's explained this twice and I'd like you to address it." There is no rule anywhere that says you must face the healthcare system alone. The people who designed it never did. Neither should you.

The System, Served

The healthcare system was not built for you. That's not your fault, and it's not a reason to give up on it — it's a reason to be strategic. Every layer of this chapter is a

strategy: the clear ask converts dismissal into a record; the second opinion converts doubt into information; the rights convert you from supplicant to customer; the referral tracker converts a maze into a checklist; the advocate converts a solo performance into a team sport. None of it requires you to become a different person — louder, more confident, more "credible." It requires preparation, documentation and persistence, which are systems, and systems can be built by anyone, including you. The first time you get the answer you need — the referral, the test, the second opinion — you'll understand what this is really about: not fighting the system, but making it serve you, the way it was always supposed to.

Toolkit Summary

- The gaslighting is a system problem, not a verdict on you: women's symptoms are dismissed as anxiety or stress, and AuDHD women are dismissed twice — for being women, and for presenting symptoms in a format the system doesn't recognise.
- A normal test result means the test was normal, nothing more. Your symptoms are real; your experience outranks a fifteen-minute dismissal.
- Self-advocacy is a skill with components — know what you want, say it in one sentence, persist, document, escalate. Scripts and repetition, not charisma.

- Use the clear-ask formula: symptom + impact + request. "I'd like that noted in my records" turns dismissal into a record; the empathy question ("what would you do for your sister?") is a last resort that works.
- A second opinion is a right, not an insult — gather information like you would for any big decision, and bring your records so the new doctor isn't starting from scratch.
- Know your universal rights: access to records, informed consent, second opinions, choosing and changing providers, and bringing someone with you.
- Seek neurodivergent-affirming providers and ask them directly about their experience — one believing doctor is worth ten convenient ones.
- Build a referral tracker (who, to whom, when, why, expected wait) with a calendar reminder to chase it. Chasing is how the maze works.
- File every insurance document in one yearly folder, keep a monthly "health admin" entry, and schedule phone calls instead of doing call-roulette.
- Bring an advocate whenever the stakes are high, you've been dismissed, or you're exhausted — there's no rule that says you face the system alone.

Chapter 10: Building Your Personal Health System

SOMEWHERE ALONG THE WAY, YOU'VE PROBABLY BEEN TOLD — BY A WELL-MEANING RELATIVE, A MAGAZINE, A DOCTOR, OR THE VOICE IN YOUR HEAD — THAT YOU NEED TO "GET ON TOP OF YOUR HEALTH." THE ADVICE THAT FOLLOWS IS ALWAYS THE SAME: EAT WELL, SLEEP EIGHT HOURS, EXERCISE, SEE THE DOCTOR REGULARLY, REDUCE STRESS. IT'S NOT WRONG. IT'S JUST USELESS, BECAUSE IT'S A LIST OF OUTCOMES WITH NO OPERATING SYSTEM UNDERNEATH THEM. TELLING AN AUDHD WOMAN TO "GET ON TOP OF HER HEALTH" IS LIKE TELLING SOMEONE TO "GET ON TOP OF THEIR FINANCES" WHILE HANDING THEM A PILE OF LOOSE RECEIPTS: THE ADVICE IS CORRECT AND COMPLETELY UNACTIONABLE. WHAT'S MISSING IS THE SYSTEM — THE CONTAINER THAT HOLDS ALL THE INDIVIDUAL PIECES, SO THAT EACH ONE HAS A PLACE, A TIME AND A TRIGGER.

This chapter is that container. Everything in this book — sleep, food, movement, hormones, appointments, medication — has been a system. This chapter builds the meta-system that holds them all: the personal health system. Not the system your doctor would design, or an app company would sell you, or an influencer would post

about. Yours. Designed around the actual shape of your brain, which means it will look different from anyone else's — and that difference is the point.

The Health Command Center

Every system in this book has the same requirement: one place where the information lives, so that nothing depends on memory. The health command center is that place for your health. It's a single location — a notebook, a folder, a notes app, a dedicated document, a wall calendar — where all the health information in your life converges. What goes in it: your medications and doses; your appointments, past and future; your doctors' names and contact details; your test results and referral letters; your symptom diary (below); your health questions as they occur to you; your yearly health map (below); and any document you'd need in an emergency. That's it. The command center doesn't need to be beautiful or organised to anyone else's standards — it needs to exist, and to be the one place you go when you need to know something about your health. The rule that makes it work is brutal simplicity: if it doesn't go in the command center, it doesn't count. You don't keep health information in your head, in a drawer, in three different apps, and on scraps of paper — you keep it in the one place, and the one place becomes your external memory, always on, always there,

never forgetting. This is the single highest-leverage health system you will ever build, because every other system in this chapter plugs into it.

The Yearly Health Map

The yearly health map is the command center's backbone: a single-page view of the year ahead, built once, that turns "I should probably see someone about..." from a vague worry into a scheduled event. Here's how to build it. Once a year — a quiet Sunday, with tea — make a list of your regular health events: the annual check-up, the dentist, the optician, the smear test or cervical screening, the medication review, the blood tests your doctor wants repeated, the specialist follow-ups, the screenings that apply to you. Put each one on the map with a month attached, based on when it's due and when it's easiest (avoid your luteal phase and your busiest work months). Then add the one-off events you know are coming — the referral you're waiting on, the procedure you've booked. The map is now your year, at a glance. To make it live, do two things. First, put the actual appointments into your calendar the moment they're booked — the map is the plan, the calendar is the execution. Second, give the map a monthly check: a standing calendar entry on the first of every month that says "Health map — what's due this month?" Five minutes: look at the map, see what's approaching, book it before it becomes urgent. The yearly health map converts

the entire annual load of health maintenance — the load that currently lives in your head as ambient guilt — into a visible, dated, done-one-at-a-time list. The guilt is replaced by a plan, and plans are executable; guilt never was.

The Daily Health Minimums

The yearly map handles the annual; the daily minimums handle the daily. The concept: define, in advance, the smallest possible version of health maintenance that keeps you running — the floor, not the ceiling — and make those the only non-negotiables. The daily minimums are the three or four things that, if they happen, mean your day is on a sound enough base that everything else is optional. For most AuDHD women they look something like: take medication; eat something real (not a perfect diet — something real); drink water; sleep, at some point, in a bed. That's it. Four items. The genius of the minimums is that they're designed to be achievable on your worst day — which is exactly when the systems need to hold. A perfect health routine that collapses on a bad day is not a system; it's a performance. The daily minimums are the reverse: they're boring, small, and almost impossible to fail, and because they're almost impossible to fail, they survive the days when everything else doesn't. Write them down. Put them in the command center. And when life goes sideways — it will — the minimums are the thing you protect, because they're the

foundation everything else sits on. On the good days, you'll do far more than the minimums, and that's fine; the minimums aren't a cap, they're a floor. But on the bad days, the minimums are the whole job, and doing the whole job — even a small job — is what keeps the system alive instead of abandoning it entirely.

The Symptom Diary That Doesn't Die

Tracking is the most powerful tool in this book, and the most likely to be abandoned, so let's be honest about why: symptom tracking dies because it becomes a second job — elaborate apps, daily guilt, colour-coded charts that stop being coloured after day four. The fix is to design the diary around your actual failure modes. First, make it tiny: one line per day, one prompt — "How was my body today?" — answered with a few words and, if you like, a one-to-five rating for pain, energy and mood. That's the whole diary. It takes thirty seconds, which is under the threshold where your brain rebels. Second, make it physical and visible: a notebook on the kitchen table, a whiteboard on the wall, a notes app with the shortcut on your home screen. The diary must live in your line of sight, or it will live in the app graveyard with all the others. Third, give it a purpose that matters: the diary exists to answer questions — "Is this medication working?" "Does this symptom have a pattern?" "Is the fatigue new?" — and a diary with a question to answer gets answered, while a diary with no purpose gets

abandoned. When a question comes up — a new symptom, a medication change, a worry — that's when you track harder for two weeks: same tiny daily entry, plus a note of the specific thing you're investigating. Two weeks of thirty-second entries is a dataset. And fourth: forgive the gaps. A missed week is not a dead diary; it's a diary with a gap. Pick it up where you left off, no guilt, no catch-up, no colour-coded apology. The diary that survives imperfection beats the perfect diary that died in week two.

The Health Budget: Money and Energy

Here's a concept the health advice industry never mentions, because it doesn't sell anything: health has a budget, and the budget has two currencies — money and energy. The money budget is the practical one: appointments, prescriptions, supplements, gym memberships, therapy. For an AuDHD brain, the trap is not overspending in a month; it's the subscriptions and memberships that auto-renew in the background of your life, silently draining a currency you can't see. The fix is a twice-yearly audit: list every health-related recurring payment, ask "am I actually using this?" and cancel what you're not. The energy budget is the one nobody budgets at all — and it's the one that matters more. Every health activity costs energy: the appointment, the phone call, the pharmacy trip, the fifteen minutes of admin, the decision about what to eat. Energy is finite, and for an AuDHD

woman it's usually already overspent by Tuesday. So budget it deliberately: when you plan health tasks, ask not just "how much will this cost in money?" but "how much energy will this cost — and what am I willing to drop to pay for it?" The health budget is why the rest of this chapter works: it's the permission to not do everything, to spread tasks across the month instead of cramming them, to choose the appointment that's a twenty-minute walk over the one that's a two-hour round trip, to say "this month, the dentist is enough." Health maintenance is not an infinite list of obligations. It's a finite budget, and you get to decide what it's spent on.

The Review Cadence: The Quarterly Check-In

Systems drift. They start sharp and slowly go blunt — the diary gets gaps, the map goes out of date, the minimums get redefined downward until they're nothing. The fix is the review cadence: a regular, scheduled check-in with yourself, where you look at the system and repair it. The quarterly check-in is the right rhythm: four times a year, on the same day (the first day of each quarter works well — it's memorable, and calendars love it), block thirty minutes and work through a short list. What's working? (Keep it.) What's not working? (Fix it, or drop it — a system that isn't working after a fair trial is not a moral failure, it's a design failure, and design failures get redesigned.) What's coming up? (Look at the health map:

what's due this quarter? Book it now.) What did my body tell me this quarter? (Scan the symptom diary for patterns.) And the question that keeps everything honest: what am I avoiding? The check-in has one job — to catch the drift before it becomes collapse — and it does it by making maintenance itself a scheduled event instead of a memory task. Thirty minutes, four times a year, is the entire cost of keeping every other system in this book alive. It is the cheapest health insurance you will ever buy.

Making It Yours: Matching Tools to Your AuDHD Profile

And now the most important instruction in this chapter, the one that all the others depend on: build it your way. Every tool in this book — the command center, the map, the diary, the check-in — has a default shape, and the default shape is a suggestion, not a law. The system only works if it fits your actual brain, and your actual brain has a profile: know it, and choose accordingly. Are you visual? Your command center should be a wall calendar and a whiteboard, not an app — you need to see it. Are you digital? A notes app with shortcuts and reminders will survive; a paper notebook will not. Do you need novelty to stay engaged? Rebuild the system quarterly (which the check-in already schedules) — a fresh layout every three months is not failure, it's fuel. Do you need sameness? Find the one format that works and never change it. Are

you all-or-nothing? Design the system in two versions — the full version for good months, the minimum version (command center + daily minimums) for bad ones. The single biggest reason systems fail for AuDHD women is that they adopt someone else's system wholesale and then conclude they're the problem when it doesn't stick. You're not the problem. The fit is the problem. Every adjustment you make — moving the diary to the wall, cutting the map to six items, changing the check-in to monthly because quarterly is too far away — is not cheating. It's engineering. It's the system becoming yours, the way a well-worn tool becomes yours.

The Capstone

This is the chapter where it all connects. The command center holds everything; the yearly map schedules it; the daily minimums anchor it; the diary feeds it information; the budget pays for it in money and energy; the check-in keeps it alive; and your hand shapes it to your brain. None of it requires you to become more disciplined, more organised, or more like a person who has never lost a prescription. It requires one afternoon of setup and four half-hours a year — and in return, you get something you may never have had: a health life that runs on systems instead of guilt, that survives your worst days, and that you can see, at a glance, in one place. You've built systems for sleep, food, movement, appointments and

medication. This is the system that holds the systems. It's yours, it's built, and it's built to last — because it was built for you.

Toolkit Summary

- The health command center: one place where all health information lives — medications, appointments, doctors, results, questions, documents. If it's not in the command center, it doesn't count.
- The yearly health map: a single-page view of the year's regular events (check-ups, screenings, reviews), built once a year, checked monthly, booked before it becomes urgent.
- The daily health minimums: the three or four smallest non-negotiables (meds, food, water, sleep) designed to survive your worst day. The floor, not the ceiling.
- The symptom diary that doesn't die: one line a day, visible, purposeful, and forgivable — a missed week is a gap, not a death.
- The health budget: audit recurring payments twice a year, and budget energy as deliberately as money — every health task costs both.

- The quarterly check-in: thirty minutes, four times a year — what's working, what's not, what's due, what am I avoiding. The cheapest health insurance you'll ever buy.
- Build it your way: match every tool to your AuDHD profile (visual or digital, novelty or sameness, all-or-nothing). Adjusting the system isn't cheating; it's engineering.
- The whole thing is a meta-system: command center holds it, map schedules it, minimums anchor it, diary feeds it, budget pays for it, check-in keeps it alive, and you shape it.

Chapter 11: Health as Self-Advocacy

IF YOU'VE READ THIS FAR, YOU'VE BUILT A LOT: SLEEP SYSTEMS, FOOD SYSTEMS, MOVEMENT SYSTEMS, APPOINTMENT SYSTEMS, MEDICATION SYSTEMS, AND THE PERSONAL HEALTH SYSTEM THAT HOLDS THEM ALL. THAT'S REAL WORK, AND IT DESERVES TO BE NAMED FOR WHAT IT IS. NONE OF IT WAS SELF-INDULGENCE. NONE OF IT WAS SELFISHNESS. EVERY SINGLE SYSTEM YOU'VE BUILT — EVERY ALARM, EVERY LIST, EVERY APPOINTMENT ACTUALLY KEPT — IS AN ACT OF SELF-ADVOCACY. IT'S YOU STANDING UP FOR YOURSELF AGAINST THE TWO GREAT ENEMIES OF YOUR WELLBEING: THE INTERNALISED VOICE THAT SAYS YOUR NEEDS DON'T MATTER, AND THE EXTERNAL SYSTEMS THAT AGREE WITH IT. THIS LAST CHAPTER IS ABOUT THE BIGGER PICTURE THAT ALL THAT WORK SITS INSIDE: WHY YOUR HEALTH IS THE FOUNDATION OF EVERYTHING YOU CARE ABOUT, WHY PRIORITISING YOURSELF IS NOT A LUXURY, AND WHY YOU DESERVE — ACTUALLY DESERVE — TO FEEL GOOD IN YOUR BODY.

Your Health Is Not Selfish

Let's start with the word that's been running your life for too long: selfish. Somewhere along the way, you absorbed the idea that taking care of yourself is taking something away from other people — that the time,

money and energy you spend on your own health is time, money and energy stolen from everyone else. This belief did not arrive by accident. It was installed, methodically, by a culture that profits from women pouring themselves out for others and never refilling. And it's wrong in a way you can prove with simple arithmetic. Think about what you actually are to the people who love you: the partner who shows up, the parent, the friend, the colleague, the daughter, the person who does the emotional labour and the remembering and the organising. Now think about what happens when you run on empty. The version of you that skips meals, postpones care and pushes through exhaustion is not a version that has more to give — it's a version that gives less: less patience, less presence, less capacity, and eventually less health, until the day comes when you have nothing left to give anyone because you're flat on your back. The math is not subtle. You cannot pour from an empty cup, and you have been trying to pour from an empty cup for years, and calling it virtue. Taking care of your health is not stealing from the people you love. It is the only way you can keep being there for them at all.

The Foundation Everything Sits On

And it's not just the people — it's everything. Your health is the foundation that every other thing in your life is built on: the work you do, the family you hold together, the creativity that matters to you, the dreams that are still

waiting. Consider what the last year of neglect cost you. The exhaustion that made you snap at the person you love. The brain fog that turned a creative project into a swamp. The appointments postponed that became crises, and the crises that ate whole weeks of your life. Every hour of "saving time" by skipping care was borrowed from a future you — and the interest is brutal. But flip it, and the picture is beautiful: every hour invested in your health pays back into everything else. Better sleep makes you a more patient parent. Managed medication makes you a more present partner. Movement makes your brain work, and a working brain is the difference between a project that dies in the drawer and one that gets finished. Regulated hormones and handled appointments mean more energy for the work that actually matters to you. This is not a trade-off — it's an investment with returns in every currency you care about. The healthier you is not a smaller version of you with less room for life. The healthier you is a bigger version — more capacity, more patience, more energy, more you — and everything and everyone you love gets the overflow.

The Permission to Prioritise Yourself

So here is the permission you have been waiting for, probably for decades: you are allowed to be the priority. Not "allowed to be a priority sometimes, when everything else is done" — allowed to be the priority, full stop, because there is no version of "everything else done" that

ever arrives. There will always be another email, another favour, another crisis, another person who needs you. If your health waits until everything else is handled, it waits forever, and you know it, because you've been living it. The permission is not a reward you earn by finishing everything; it's a decision you make now, with things unfinished — which is how life always is. Let's be precise about what this permission does and doesn't mean. It doesn't mean abandoning your responsibilities or becoming a person who only cares about herself. It means changing the order of operations: your health first, not because you're the most important person in the room, but because you're the only person who can carry you, and everything you carry for others depends on that. It means the doctor's appointment goes in the calendar before the favour you were going to do for a colleague. It means the sleep is non-negotiable even when someone is disappointed. It means "I can't, I have a health appointment" is a complete sentence, said without apology, because it is a true sentence and true sentences don't need apologies. The people who love you will adjust. The people who don't love you — the ones who benefit from you running on empty — will push back, and their pushback is not evidence that you're doing something wrong. It's evidence that you're finally doing something right.

Health as Self-Respect, Not Self-Indulgence

There's a version of self-care that has been sold to women, and it's worth rejecting it by name: the candle-and-bubble-bath version, the spa day, the retail therapy, the treat-yourself consumerism that mistakes spending for caring and treats health as a reward for enduring life rather than a foundation for living it. None of that is what this book is about, and I want you to know the difference, because the difference is what makes the whole project sustainable. Self-indulgence is comfort without purpose — it feels good and changes nothing. Self-respect is care with purpose — it may not feel like a treat at all, and it changes everything. Refilling the prescription is self-respect; it is boring, unglamorous, and it is love for yourself made visible. Booking the follow-up, taking the medication, going to bed at a reasonable hour, eating something real, keeping the appointment you didn't want to keep — these are not indulgences. They are the daily practice of treating yourself as someone who matters. And that practice is the quietest form of self-advocacy there is, because it doesn't require a single confrontation. Every time you choose the boring, necessary, unglamorous act of care, you are voting — with your time, your energy and your money — for the proposition that you are worth taking care of. Vote often enough, and the proposition stops being a belief you're trying to hold and becomes a fact you're simply living.

Every System Is an Act of Self-Advocacy

Here's the reframe that ties the whole book together: the systems you've built are not just logistics — they're advocacy, written in the language of alarms and lists. Think about what an advocate does: they make sure your needs are heard, they keep records, they persist when they're dismissed, they show up prepared, they bring support. Now look at what you've built. The symptom list is your voice, written down so it can't be interrupted. The question script is your advocate, in your pocket. The medication check is your persistence, scheduled monthly. The written summary request is your record-keeping. The referral tracker is your follow-up, automated. The health command center is your case file. The daily minimums are your bottom line. You have built, piece by piece, the most effective advocacy apparatus a person can have — and you built it for the person who needed it most, which is you. This matters more than you may realise, because it means you don't need to become a different person to advocate for yourself. The loud, confident, never-doubting advocate is a myth; the real advocates are the ones with the lists, the records, the alarms and the persistence — and that is exactly what you have built. Every alarm that fires is a small act of defiance against a world that expected you to forget. Every appointment kept is a correction of a system that expected you not to come. Every refill on time is proof that the person who once

couldn't manage her own medication now manages it with a system — and a person who can do that can do anything.

The Future: More Capacity for the Life You Want

Let's look forward now, because that's where this all points. Picture yourself a year from now, with the systems running — not perfectly, because nothing runs perfectly, but running: the meds taken, the appointments kept, the sleep defended, the food real, the body less often an emergency. What does that version of you have that you don't? Capacity. That's the whole return on this investment: not a perfect body or a flawless routine, but more capacity — more energy in the tank, more patience in reserve, more working memory free, more of you available for the life you actually want. The life you want is not a life of doctor's appointments and pill organisers; those are just the scaffolding. The life you want is the one that becomes possible when the scaffolding holds: the creative project that finally gets finished, the relationship that gets the version of you who isn't running on fumes, the plans that stopped being made because you were too tired to make them. Health is not the point of your life. It's the thing that lets you get to the point of your life. And you don't have to wait for a perfect system to start living that life — you've been building toward it through

every chapter of this book, and the building is the living. The systems are the groundwork; the capacity they free up is the point.

The Final Word

So here it is, the final word of this book, and I want you to read it slowly: you deserve to feel good in your body. Not "you deserve to feel good in your body once you've earned it, once you've fixed everything, once you're a better person." You deserve it now, as you are, with your AuDHD brain and your imperfect systems and your pile of unfinished everything. Feeling good in your body is not a reward for being a good woman. It is your birthright, as much as it is anyone's. The world has spent your whole life telling you otherwise — that your discomfort is normal, that your needs are too much, that your body is a problem to be managed or ignored, that you should be grateful for whatever care you can get. Every one of those messages was wrong. Your body is not the problem. Your body is the only home you will ever live in, and you are allowed — you are encouraged, you are entitled — to make it a good one. The systems in this book are how. The belief underneath them is why: you matter. Not the version of you that could be more organised, more disciplined, more like someone else — this you, exactly as you are. You have always deserved to feel good in your body. The only thing that's changed is that now you know

it, and now you have the tools. Go and use them. Your health is your advocacy. Your body is your home. And you deserve to feel good in it — starting now.

Toolkit Summary

- Your health is not selfish: you cannot pour from an empty cup, and the version of you that runs on empty gives less to everyone who loves you. Care for yourself is how you stay able to care for them.
- Your health is the foundation everything else sits on — work, family, creativity. Every hour invested in it pays back in every currency you care about.
- You are allowed to be the priority, with things unfinished — that's how life always is. "I can't, I have a health appointment" is a complete sentence.
- Health is self-respect, not self-indulgence: the boring, unglamorous acts of care — the refill, the follow-up, the early night — are love for yourself made visible.
- Every system you've built is advocacy in another language: the symptom list is your voice, the question script is your advocate, the tracker is your persistence, the command center is your case file.

- The real advocates are the ones with the lists and the alarms — and that is exactly what you've become.
- A year of systems running buys you one thing that changes everything: capacity — more energy, more patience, more of you available for the life you want.
- The final word: you deserve to feel good in your body, now, as you are. Your health is your advocacy. Your body is your home. And you deserve to feel good in it — starting now.

Resources and Further Reading

THIS SECTION IS A MENU, NOT A SYLLABUS. YOU ARE NOT REQUIRED TO READ EVERYTHING HERE, WORK THROUGH IT IN ORDER, OR FINISH ANY OF IT. TRY WHAT CALLS TO YOU, SKIP WHAT DOESN'T, AND RETURN TO WHAT HELPS. BOOKS AND WEBSITES GO OUT OF PRINT AND MOVE; IF A LINK YOU FIND HERE HAS CHANGED, SEARCH THE TITLE OR THE ORGANIZATION'S NAME AND YOU'LL USUALLY FIND ITS NEW HOME. CONTENT NOTES: SOME OF THESE RESOURCES DISCUSS TRAUMA, MEDICAL GASLIGHTING, AND MENTAL HEALTH CRISES IN DETAIL. TAKE THEM IN DOSES, THE WAY YOU TAKE EVERYTHING ELSE NOW.

Books

The Autistic Brain: Thinking Across the Spectrum, by Temple Grandin and Richard Panek. Grandin — the most famous autistic adult in the world — walks through how autistic brains actually work: the sensory systems, the thinking styles, the strengths and the struggles. It's a foundational book for understanding your own hardware, and Grandin's practical, engineer's-eye view of the brain is a refreshing antidote to the tragedy narratives that still dominate autism writing.

ADHD 2.0: New Science and Essential Strategies for Thriving with Distraction, by Edward Hallowell and John Ratey. Two of the most respected ADHD clinicians in the world, writing for adults, with a strongly affirming stance: ADHD as a trait, not a defect. It covers the newest brain science and practical strategies, and it's especially good on the emotional side of ADHD — the sensitivity, the shame, the rejection dysphoria — that the symptom lists never capture. Read it alongside this book; they agree with each other.

Burnout: The Secret to Unlocking the Stress Cycle, by Emily Nagoski and Amelia Nagoski. The most practical book on stress and exhaustion you'll find. The sisters' central insight — that stress is a cycle that must be completed, not just endured — explains why rest alone often isn't enough, and why your body keeps re-running the alarm even when the danger is past. Written with warmth, humor, and zero blame, it belongs on the shelf of every AuDHD woman who has ever been told she's "just stressed."

The Body Keeps the Score: Brain, Mind, and Body in the Healing of Trauma, by Bessel van der Kolk. The landmark book on how stress and trauma live in the body — and why you can't always think your way out of what your nervous system is holding. Relevant here because health neglect and medical gaslighting leave marks on the body, not just the mind. Content note: it includes detailed accounts of abuse and trauma that some readers find heavy, and parts of the clinical picture are debated;

the core message — that the body registers what the mind tries to dismiss — is what matters. Read it in doses, or read summaries and interviews with the author if the full book is too much.

A few more that belong on the shelf: Unmasking Autism by Devon Price — the book that has changed more late-diagnosed lives than any other; Invisible Women by Caroline Criado Perez — how the world, including medicine, was designed around men, and what that costs women's health; How to Keep House While Drowning by KC Davis — a short, shame-free guide to care tasks; and The Menopause Manifesto by Dr. Jen Gunter — a clear, myth-busting guide to the hormonal transition that hits AuDHD brains so hard.

Sleep Resources

Sleep is the foundation this book keeps returning to, and the resources for AuDHD sleep fall into three useful buckets. **The science:** the Sleep Foundation (sleepfoundation.org) and the National Sleep Foundation both publish accessible, evidence-based guides to sleep problems and their causes — start there if you want to understand the mechanics before you change anything. **The AuDHD-specific practical stuff:** ADDitude magazine (additudemag.com) has a deep archive of articles on ADHD and sleep — delayed sleep phase, racing minds at bedtime, the sleep-medication interaction, and how to actually build a wind-down

routine when your brain refuses to wind down. Search "sleep" on the site and you'll have reading for a month. **The structured programs:** cognitive behavioural therapy for insomnia (CBT-I) is the most evidence-backed treatment for chronic insomnia, and it's available as structured digital programs (Sleepio is the best known) and through many health services — it suits our brains well because it's a system, not a vibe. Two AuDHD-specific cautions: melatonin and other sleep aids interact with ADHD medication and other prescriptions, so run anything past your prescriber; and the classic advice to "just relax" before bed is useless for a brain that relaxes on its own schedule — the systems in this book, not relaxation, are the answer.

Nutrition Resources for ADHD

The chapter on food made the case that eating is an executive function task. The resources here follow the same principle: practical, shame-free, and built for brains that can't do elaborate meal planning. ADDitude's nutrition section is the best single starting point — search "meal planning" and "protein" for articles on the protein-rich breakfast that supports medication effectiveness, on low-effort meal systems, and on the executive-function case for eating the same safe foods repeatedly. For individualised support, a growing number of dietitians specialise in ADHD and neurodivergent eating — the Academy of Nutrition and Dietetics (eatright.org, US) and

the British Dietetic Association (UK) both have find-a-dietitian tools. Look for practitioners who describe themselves as neurodivergent-affirming and who work with low-effort, safe-food-friendly, anti-diet approaches rather than "optimal nutrition" perfectionism. And keep How to Keep House While Drowning close: KC Davis's approach to "care tasks" — food included — is the most AuDHD-compatible nutrition philosophy in print, because it starts from the premise that feeding yourself is an act of kindness, not a performance.

Exercise Resources

Movement for an AuDHD brain needs to be motivation-free, which means the resources that help are the ones that make moving easy, not the ones that make you feel bad about not moving. **The science that motivates:** Spark: The Revolutionary New Science of Exercise and the Brain by John Ratey (of ADHD 2.0 fame) documents how exercise directly improves attention, mood and executive function — it's the best evidence that movement is ADHD medication-adjacent, not a chore. **The practical frameworks:** ADDitude's exercise section covers "exercise snacks," body-doubling for workouts, and how to choose movement you'll actually repeat; the anti-diet, joy-focused movement community (search "joyful movement" and "intuitive exercise") is a healthier starting point than gym culture for anyone who has ever felt shamed by fitness content. **The low-friction options:**

free workout apps that require no booking and no social performance (Nike Training Club and the built-in apps on most phones are good starting points), and — for the sensory-averse — swimming and other water-based movement, which many autistic people find far more tolerable than gyms. The rule from this book applies to all of it: the best exercise is the one that survives your worst day, which usually means the smallest one.

Hormone Health Resources

The hormonal layer of AuDHD health is the most underserved, so the resources here matter. **For the cycle:** the International Association for Premenstrual Disorders (IAPMD.org) is the authoritative, evidence-based organization for PMDD and severe PMS — the very conditions that hit AuDHD women disproportionately. Their symptom trackers and clinician-finder are exactly what you need to document the pattern the chapter on hormones described. **For perimenopause and menopause:** the Menopause Society (menopause.org, formerly NAMS) publishes reliable, non-alarmist information; the Balance app (balance-menopause.com), created by menopause specialist Dr. Louise Newson, offers free, detailed perimenopause tracking and information; and The Menopause Manifesto by Dr. Jen Gunter is the definitive myth-busting book — read it before your forties so you know what's coming. **For the brain connection:** search "oestrogen ADHD" on

ADDitude and Neurodivergent Insights (neurodivergentinsights.com) for the growing body of practical writing on how hormonal shifts affect AuDHD symptoms, medication effectiveness, and PMDD — the same conversation this book raised, in more depth.

Medical Advocacy Organizations

You are not the only one fighting this fight, and the organizations below exist to hand you tools, information and sometimes a literal advocate. **Autistic Women & Nonbinary Network (AWN, awnnetwork.org)** — a community organization built by and for autistic women and nonbinary people, with essays, resources and a strong record on health advocacy from a neurodivergent perspective. **Autistic Self Advocacy Network (ASAN, autisticadvocacy.org)** — the leading autistic-led advocacy organization; their health-care resources are written by autistic people and are unusually clear about patients' rights. **National Patient Advocate Foundation (patientadvocate.org)** — a US organization that helps patients navigate insurance, appeals, denials and access to care, including a professional case-management service; if you're fighting a denial or a maze of paperwork, this is what they're for. In the UK, the National Autistic Society (autism.org.uk) and ADHD UK (adhduk.co.uk) both maintain health and rights guidance, and Patient.info has solid plain-language explainers on NHS rights and complaints pathways. In other countries,

search "patient advocacy [your country]" — most systems have some equivalent, and knowing who they are before you need them is half the battle.

Apps

Apps are tools, not saviours — the right one can carry a system, the wrong one can become another abandoned icon. **Medication reminders:** Medisafe and MyTherapy are the most reliable and privacy-conscious medication apps — reminders, refill tracking, missed-dose logging, and data you can export to your prescriber. **Symptom and health tracking:** Bearable is the standout for AuDHD users — it tracks symptoms, mood, energy, sleep, medication and factors in one daily check-in, and produces exactly the pattern reports a prescriber can act on. Daylio is a lighter, mood-and-activity option if Bearable feels like too much. **Cycle tracking:** Clue is the most data-focused and privacy-respecting period app — its prediction is built on your data, not on advertising — and it has a PMDD tracking mode that pairs with the IAPMD tools above. **A caution, offered with love:** for every app you add, you add a daily obligation. Start with one — the one that solves your worst problem — and add another only when the first has survived a month. One app you actually use beats five you feel guilty about.

Online Communities

Community is medicine, and it's also a place to get hurt, so choose carefully. The most active and useful spaces for AuDHD women's health questions are on Reddit: **r/AuDHD**, **r/ADHDWomen**, **r/AutismInWomen** and **r/PMDD** all have rich, practical threads — people describing the exact symptoms you've been trying to name, the medication interactions your doctor never mentioned, the sensory-friendly dentist recommendations, the shortage survival strategies. Search each for your specific question before you post — someone has usually already asked it. Facebook groups for late-diagnosed AuDHD women exist in every country and are wildly variable in quality; look for ones that moderate kindly and ban pseudo-science and miracle cures. Two cautions, offered with love. First, communities can become a second job: the endless scrolling, the comparison, the urge to help everyone who posts in worse shape than you. Use them for companionship, not as a new obligation, and step away whenever they spike your anxiety rather than soothing it. Second, you will read other people's worst days — including posts from people in crisis. If that lands hard, that's information about what you can absorb right now, not a reason to push through it. Your health comes before your community service, always.

Crisis Resources

If you are in immediate danger, call your local emergency number — 999 in the UK, 911 in the US and Canada, 112 across much of Europe — or go to your nearest emergency department. If you're in the UK, **Samaritans** (116 123, free, 24/7, from any phone) listen without judgment. In the US and Canada, **988** connects you to the Suicide & Crisis Lifeline, 24/7. For help finding a helpline in any other country, **findahelpline.com** maintains a searchable international directory of crisis lines and text services. Crisis support exists so that you don't have to hold the worst moments alone. Using it is not failure. It is exactly what it's for, and you deserve the same care you'd urge on a friend. And if you need a non-emergency health answer — a medication question, a worrying symptom, a prescription problem — most health systems have non-emergency lines and pharmacy services precisely so you don't have to wait and worry alone.

Neurodivergent-Affirming Provider Directories

Finding providers who believe you is the highest-leverage health decision you can make, and the directories below are where to start. **NDTherapists.com** — a directory specifically of neurodivergent-affirming therapists, built with the community it serves. **Inclusive Therapists** — centers neurodivergent and marginalized

identities, with detailed filters. **Psychology Today's** therapist directory lets you filter by specialty — search "autism," "ADHD," "AuDHD" in your area. For medical providers (doctors, dentists, gynaecologists), there's no universal directory yet — the best tools are word of mouth in the communities above ("does anyone know an ADHD-literate GP?" threads produce real answers) and the AWN and ASAN resource lists, which maintain vetted providers in some regions. The screening question from the chapter on the health system applies everywhere: ask any prospective provider, directly, "Have you worked with autistic and ADHD adults? What's your approach?" — and trust what the answer tells you.

One final note. You will meet professionals — and authors, and well-meaning relatives — who tell you to try harder, push through, or just think positively. They are describing a body and brain that are not yours, and you are under no obligation to be educated by them. You know more about your health now than most of them ever will. These resources are here to support you, not to replace your own growing knowledge of yourself. You are the expert on your own body. Everything else is just company on the way.

Toolkit Summary

- Treat this list as a menu, not a syllabus: try what calls to you, skip what doesn't, take heavy material in doses.

- Start with the books: *The Autistic Brain*, *ADHD 2.0*, *Burnout*, and *The Body Keeps the Score* (in cautious doses) — plus *Unmasking Autism*, *Invisible Women*, *How to Keep House While Drowning*, and *The Menopause Manifesto* when you're ready.
- Sleep: Sleep Foundation for the science, ADDitude's sleep archive for the AuDHD specifics, and structured CBT-I programs for chronic insomnia.
- Nutrition: ADDitude's nutrition section, neurodivergent-affirming dietitians, and KC Davis's shame-free care-task philosophy.
- Exercise: *Spark* by John Ratey for motivation, ADDitude's exercise-snack frameworks, and low-friction options that survive your worst day.
- Hormones: IAPMD for PMDD and cycle tracking, the Menopause Society and Balance app for perimenopause, and the growing AuDHD-hormone writing on ADDitude and Neurodivergent Insights.
- Advocacy organizations: Autistic Women & Nonbinary Network, ASAN, and the National Patient Advocate Foundation (US) — plus NHS/ rights guidance for the UK.
- Apps: Medisafe or MyTherapy for medication, Bearable for symptoms, Clue for cycles — and add apps one at a time, only when the first has survived a month.

- Use online communities for companionship, choose well-moderated spaces, and step away when they spike your anxiety.
- Know your crisis resources before you need them: Samaritans (116 123, UK), 988 (US/Canada), findahelpline.com for everywhere else.
- Find neurodivergent-affirming providers through NDTherapists, Inclusive Therapists, and word of mouth — and ask every provider the screening question directly.
- The best resource is your own growing knowledge of yourself. Everything here is company on the way.

About the Author

Clara Bennett is a writer and advocate specialising in the lived experience of neurodivergent women — the overwhelm, the burnout, the health battles that come with a brain that does everything at full volume.

Her work focuses on translating research and lived experience into practical systems that survive contact with a real week. She writes for the women who have been told to just try harder, and who deserve better than that advice.

Clara lives in the UK and is the author of the Fix series: practical guides for ADHD and AuDHD women.

Also by Elena Voss, MSc

ADHD in Women: The Late Diagnosis Guide

The AuDHD Woman: Understanding Autism and ADHD
as a Dual Reality

Anxious Woman's Compass: Navigating Anxiety with
Practical Tools

The Autism Toolkit for Women: Practical Strategies for
Navigating Daily Life, Work, and Relationships

The Weather Within: A Bipolar Guide for Women

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